Brain Science, Nerves, and Clinical Psychiatry

Nelson Sizer

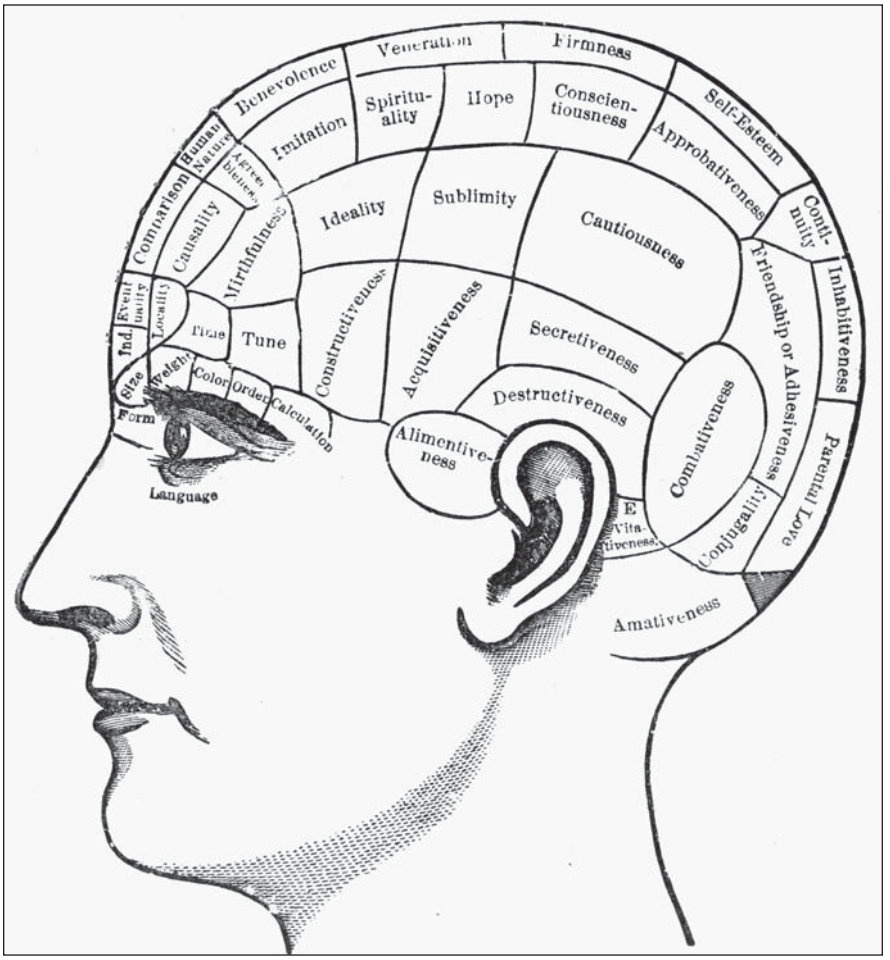
(1812–1897)

Forty Years in Phrenology: Embracing Recollections of History, Anecdote, and Experience

(1891)

Although today it is often dismissed as little more than quackery, phrenology—the study of the shape and size of crania and how these relate to character attributes—was a serious discipline in the nineteenth century and a precursor to the field of neurology. Its founder, the Badenese anatomist and physician Franz Joseph Gall (1758–1828), observed brain-damaged patients and came to the conclusion that specific aptitudes, inclinations, and faculties were associated with certain parts of the brain, which, in turn, left peculiar traces on the skull.

Phrenologists, however, not only were involved in research. They practiced as itinerant lecturers, psychological examiners, and advisers. In the United States, one of the most successful group of phrenologists were the siblings Orson, Charlotte, and Lorenzo Fowler, who published a prominent journal on the subject and founded consulting firms in major cities throughout the Northeast. From 1838 until the turn of the century, they and their employees



A model phrenological head. From Nelson Sizer and H. S. Drayton, *Heads and Faces, and How to Study Them: A Manual of Phrenology and Physiognomy for the People* (New York: Fowler & Wells, 1887), 195.

visited small towns, renting local churches or town halls and giving lectures on phrenology. Along the way, they were often asked to serve as counselors to parents, spouses, and business associates. The following excerpt from the memoirs of the practitioner Nelson Sizer, an employee of the Fowlers, gives us a sense of the ways in which phrenologists served as incipient psychotherapists.

Timid Child Managed—a Great Test

Another instance occurred in this town, the recital of which may serve to aid some mother or teacher in the management of an unduly cautious child. At the close of a lecture on the nature and training of the sentiments of Approbativeness and Cautiousness, in which I had said that half the trouble which people had with timid children was largely owing to their improper management; adding that however much afraid of strangers any bright, intelligent child, two or three years old, might be, I would undertake to get it willingly into my lap in twenty minutes. A bright and genial lady came up to the platform and said to me, "I have a boy two and a half years old, that I think is bright, but he has never been in the lap of any person not belonging to the family; even his grandpa, who has been in and out almost daily for the last year, can make no headway in overcoming the child's aversion to strangers. Now, if you will come into my house and get that boy into your lap, willingly, in twenty minutes or twenty hours, I will believe in Phrenology."

I found out where she lived and arranged to go there at one o'clock the next day and to enter the dining room in the extension of the house, without knocking and that neither she nor her husband should say any person was coming, or look at or say a word to "Charlie" when I came, nor while I stayed, and I was not to be treated as a stranger while there.

At the hour appointed I entered the house, the family was at the table. Charlie slipped out of his chair and left for the kitchen as quick as legs and "wings" could carry him. I instantly spoke in a tone of familiarity to the parents: "What made you eat up all the dinner so that I can have none? I will pick what I can get." I took a seat at the table and began to eat—and kept talking in a way that a child, which I felt certain was listening, would understand—then laying one hand on the father's head and the other on the mother's, kept on telling them what they were fond of and what they could do, and stealthily turning toward the open door into the kitchen, saw about half of the little head and one bright eye peeping around the door jamb, of course wondering who and what that stranger could be who seemed so much at home with the house, the dinner, and the parents. I went on examining the heads and talking, keeping my back toward the little spectator, who forgot that I saw him leave the room, and, perhaps supposed I did not know he had existence. He edged

his way into the room, and as he was against the wall quite a distance from the door, I kept turning my back toward and my face directly from him so as to compel him to get very near me before he could see the face of the drollest man that ever he saw in his home. Of course the plan was to ignore the boy, yet to talk so that he could comprehend it. All at once I walked away from the boy to the opposite side of the room and looking up to a gaudy picture, representing Solomon's temple, with the Sanhedrin in session wearing their red robes, I said, "What a splendid picture Charlie has here!" and then I kept on describing the figures of the council and calling them men and ladies and boys, and I dropped my eye and he stood by my side eagerly looking to learn, for the first time, the mysteries of the great picture which, the stranger had said, was Charlie's. He had forgotten that I was a stranger in the sense of being dangerous. I had said nothing to him, had not looked at him, had not tried to have him come to me, but had let him alone, and talked steadily about what he could not understand, and he had got all the faculties of curiosity aroused, and his Cautiousness had gone to sleep.

I stooped and picked him up saying, "You can't half see it down there, I will show you all about it." And his finger on the picture with mine trying to tell me what he could of its new-found beauties. The fact that it was *his*, was a new thing to him, and I seemed to him to know more about his interests and possessions than his mother did.

I then set him down, for fear it would occur to him that I was a stranger, and walked right away from him and went where his father and mother sat, marked off a chart for the mother, and the boy was leaning against me, apparently very much at home, and trying to be interested in what I was doing. I opened my chart, which contained pictures, and told Charlie if he wanted to see the picture he might come now, and he climbed into my lap without assistance, while I kept the pictures of the book out of the reach of his eyes until he had got fairly into my lap. It was a struggle, and when he got fixed and gave a sigh of contentment, I turned toward the blazing and half tearful eyes of the mother interrogatively, and she burst out, "I give it up. Oh, how did you do it?"

I quietly replied, "I made no appeal to his Cautiousness, but did everything to allay that feeling, and to awaken curiosity and excite his judgment, imagination, and affection. Ignoring him was just what he needed, yet it was what others did not do, and you always tried to urge him to pay attention

to the stranger, and make friends with him. That defeated its own purpose. I took a different course, and you see the result."

The boy talked of me for months afterward, and wanted me to "come some more." This method of curing timidity I always use when necessary, and it is wonderful how quickly other faculties can be awakened, and Cautiousness be allayed. A timid child is talked to and coaxed by every one that calls, and so grows worse. If left alone and unnoticed, it would soon get over its bashfulness.

"A Spoiled Child, and How It Was Done."

"Mistaken severity as well as mistaken kindness will equally, but very differently, spoil a child. As over-indulgence in every whim or imaginary want of a child leads to effeminacy, amiable selfishness, capricious exactions from friends and servants, and a general helplessness; so, on the other hand, too much strictness and severity in the training ruins the temper and makes a vixen to torture the next generation, or utterly crushes the spirit and makes life to the child a 'vale of tears.' We give a case in point:

"I examined in this place the head of a little girl four years old, and found Destructiveness and Combativeness very largely developed. Wondering why these organs should be so large, I referred to the heads of the father and mother and a younger child, and found that none of them had those organs in more than a medium degree. This, of course, excited my surprise, and I felt it necessary to account for the discrepancy, or ascertain the history of the case. Accordingly, I suggested to the parent that the child must have been very much annoyed and irritated by surrounding influences to induce at so early an age such extraordinary developments.

"The mother, with regretful earnestness, replied: 'That is true, and I will explain the reason. I have been a teacher and "boarded around," and seeing much slackness and imbecility in parental government, I firmly resolved if I ever had children, I would begin with them in season and make them go straight. Accordingly, this girl being my first child, I began early to make her toe the mark, and I used to train and whip her for every little offense or neglect. She has become fretful, peevish, and violent in temper, so that now, whipping only makes her worse. A few days ago I lost my temper and gave her a severe whipping, and the moment I got through with her she seized the fire-tongs, and with a severe blow she broke the back of her

pet kitten that was sitting by the fire. When her anger had subsided she mourned piteously for the death of her pet, and she can not get over her loss. She is a very bad child when angry, and I do not know what I can do with her. I have, however, taken a very different course with my other one, and she is easily managed, though her natural disposition is no more amiable than that of the older one was at first. I fear I have spoiled my little girl by unnecessary strictness and severity.'"

This painful fact has doubtless since then helped me in hundreds of instances, to guide and aid other mothers in the adoption of better methods in the training of their precious pets, whose upgrowth to goodness and to God was the hope and the burden of their life. . . .

Insanity Cured by Phrenology

While here, I received a call from a friend residing twelve miles distant, at Suffield, Conn., where I married my wife and resided during 1843. He informed me that Henry Bissell, of Suffield, had recently received a blow upon the head in the region of the temple, and had become insane in consequence. He appeared somewhat strangely for a day or two, and then took the train for New York, and before arriving there, attracted attention by immoderate laughter at everybody and everything in the car. A gentleman who knew him happened to be on the train, and took him back to Hartford, left him in the asylum, and sent for his father. Here he had been for several weeks under treatment without any apparent benefit. On hearing these facts I wrote at once to the father, and sent it by my informant, stating the impression that the injury was upon the seat of Mirthfulness, hence his tendency to laugh and see absurdity in everything, and suggested that if the physician would apply leeches and ice to that part of the head which was injured, the symptoms of insanity would cease. The aged father, who was interested in our lectures on Phrenology at Suffield in 1841, recognizing the reasonableness of the views that I had taken of the cause and proper treatment of the case, on receiving my letter at eight o'clock that night he instantly harnessed his team for a dreary drive of seventeen miles to Hartford, and, reaching the asylum at eleven o'clock, after Dr. Butler had retired, he insisted upon seeing him at once. With my letter open in his hand, the anxious father met the doctor, who read it deliberately and said: "It looks reasonable, and we will try the treatment in the morning."

"No, doctor; we will try it to-night, if you please. I can not wait till morning."

"All right," said the doctor, "to-night, if you say so."

In half an hour the patient was under the treatment of leeches, in another half hour the injured part was under the influence of pounded ice, and he was fast asleep. The next morning he and his father took breakfast with the doctor; "he was clothed and in his right mind," and in a short time went home with his father, apparently cured.

The injury was directly over the organ of Mirthfulness, and the inflammation caused by the blow, produced the deranged action of that faculty. Thirty-seven years have now elapsed, since this injury was received and cured, and there has been no return of the symptoms of insanity. Had the inflammation been allowed to proceed, death, or mental derangement for life, might have been the consequence. The young man being my friend, I felt peculiar interest in the case.

Thus Phrenology throws a flood of light on the subject of insanity for those who wish to learn.

Trades Selected for Boys

"I am from Patterson, sir; you will remember I brought to you my three elder boys, and you selected for each of them the trade he was best fitted for, and they are thriving at them nicely, and say they could not and would not change trades on any account. They often talk about it, and each boasts over the other that he has the best trade of the lot. Now I have brought this one, and I shall put him to the trade you say he is best adapted to. I believe you know about it, for you have placed the other three so well, and the trades are so different; only think one is a jeweler, one is a butcher, and the other is a carpenter. I have one more besides this, and I shall bring him when he is old enough to put to business."

And this is not the only family who is doing a similar thing, and such work serves to keep us up to a sense of our responsibility.

From Nelson Sizer, *Forty Years in Phrenology: Embracing Recollections of History, Anecdote, and Experience* (New York: Fowler and Wells, 1891), 63–67, 73–74, 188–189, 295–296.

George Miller Beard
(1839–1883)

Cases of Hysteria, Neurasthenia, Spinal Irritation, or Allied Affections
(1874)

Similar to scholars in the eighteenth century, observers on both sides of the Atlantic during the last third of the nineteenth century came to believe that nerves played a pivotal role in human health. The brain was often compared to a battery producing electricity that passed its energy through nerve fibres in a manner akin to the way in which electrical wires function. Too much or too little nervous energy, it was believed, could lead to a disequilibrium that came to be known as a "nervous breakdown."

George Miller Beard became one of the most famous proponents of this view. Beard began his career as an electrotherapist, following a tradition, which went back to the eighteenth century, of administering electrical current to individuals suffering from any number of different ailments, including paralysis, rheumatism, indigestion, headaches, and impotence. In 1869, he published an article announcing a new diagnosis, neurasthenia, a disease he believed was caused by the hectic pace and demands of modern life.

Under this head I propose to detail a few cases of nervous disease—commonly called functional—that are at once exceedingly frequent and exceedingly annoying both to patients and physicians. I shall treat the subject mainly from the clinical and practical stand-point, reserving the discussion of its scientific and philosophic relations for another occasion.

By hysteria and allied affections I mean that large and increasingly numerous class of affections that pass among the people and among the profession by the vague and half-erroneous terms, spinal irritation, nervous exhaustion, general debility, general neuralgia, etc.

In a work in which I have long engaged, and which is now slowly progressing, I hope to be able to unify these diseases—to show that they have in general a common pathology, a common history, a common group of symptoms, and a common therapeutics. I shall seek to show that these diseases, or symptoms of diseases—or, as they might, perhaps, with better justice be called, results of disease—are expressions of a common nervous diathesis; that they are all liable to run into each other, and to act vicariously to each other; that they are part of the price we pay for civilization, being confined mostly to the enlightened peoples of modern times; and that they are, in all their dreary shapes, most abundant in the northern portions of the United States of America. . . .

General Principles of Treatment

I treat all these affections, by whatever name known, on the same general principles, varying and adapting the method according to individual need. Of the various methods of using electricity, I depend mainly on general faradization and central galvanization, using sometimes in alternation—in some cases finding the former, in others the latter, more beneficial.*

Internally, I use preparations of phosphorous and cod-liver oil, and sometimes arsenic. I make large use of the cod-liver oil emulsion. I have seen good results from the oxide and phosphide of zinc and chemical food.

Externally, I use ice and hot-water bags to the spine with studious caution, and mild and cautious counter-irritation to tenders points on the spine. My method of counter-irritating nervous patients is, to take one of Alcock's porous plasters and cut off a piece of about the size and shape of my little finger;† along the centre of this I place a little Spanish-fly ointment, and then apply over the tender spot, and let it stay there until it falls off. Counter-irritation thus used is not very annoying, and is quite effective. I use Alcock's porous plaster because it sticks better than anything I can find.

Except when I am experimenting, I use all these remedies, or several of them, simultaneously.

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*Editor's note: Faradization involved applying alternating current, while galvanization referred to the application of continuous, direct current.

†Editor's note: Alcock's Porous Plasters were adhesives purported to have electrifying properties that could alter blood circulation.

Neurasthenia, Cerebrasthenia, Myelasthenia

The old and forgotten term, *neurasthenia*, I have for several years applied to the condition known in common language as *nervous exhaustion*; and I have recently subdivided this condition into *cerebrasthenia* and *myelasthenia*, according as the exhaustion is chiefly manifested in the brain or in the spinal cord. When the exhaustion shows itself chiefly in the brain, there are the symptoms of insomnia, headache, vertigo, flashes before the eyes, muscae volitantes, tinnitus, etc. When the exhaustion shows itself chiefly in the spinal cords, there are the symptoms of pain in the back, at any point below the first cervical and last dorsal vertebrae, and mostly between the shoulder and in the lumbar region; spinal tenderness (though not always); weakness of the lower limbs, and sometimes of the arms; flatulence; feeling of oppression on the chest; gastralgia, intercostal and abdominal; neuralgia of the bladder and sexual disturbance; numbness of the extremities, etc. While the term neurasthenia implies both cerebrasthenia and myelasthenia, yet in some cases the exhaustion seems to be almost exclusively confined to the brain alone, or to the spinal cord alone. . . . The meaning of these terms will be made more clear by the following cases:

Case I. Mr. L————,aged 86, was referred to me, June 25, 1873, by Dr. Geo. Baker. For several months, since January 1873, the patient had suffered from vertigo, feeling of tingling, pricking, and stinging over the surface of the body; pain in the back; dyspepsia; constipation, insomnia, and mental depression. The spinal irritation was quite variable in its seat, being sometimes in the lower, sometimes in the upper vertebrae. Sometimes there was tenderness of the cervical vertebrae, with stiffness of the neck.

The diagnosis was neurasthenia, including cerebrasthenia and myelasthenia; and the pretty evident cause was excess in sexual indulgence combined with over work in business.

I gave general faradization alternately with central galvanization, for one month; and, at the same time, used phosphide of zinc pills in doses of 1–10 of a grain, and chemical food. Counter-irritation was also employed over the tender vertebrae. July 5, he was much better. August 1, still better; and September 1 he resumed active business.

Case IV. Miss G————, a young lady of about 24 years of age, was first seen, with D.O.L. Mitchell, Nov. 23, 1871. The patient was of a very fine organization, and of slight, fragile build. Left an orphan at an early age,

she had worked hard as a copyist in a telegraph office, where she toiled many hours a day to support herself and her younger brothers and sisters. For two years or more she had been in a condition of excessive debility, which her physician could control only imperfectly by medication.

She could not walk a single block, or even part of a block, and so remained constantly indoors. Her appetite was feeble and fickle; sleep was uncertain and disturbed; the circulation unequal. The pulse, though weak and nervous, was yet tolerably strong for a delicate lady, but was very susceptible to mental influence. Careful examinations had been able to detect no disease of the lungs or heart or of any organ; the uterus had not been examined.

Fainting spells, or spells resembling fainting, came over her after severe exertion; even the *shock* of *hearing* the door open fearfully agitated her, so that she suffered for a number of minutes. She dreaded the coming of a new doctor, and lived in a condition of painful apprehension when she learned that I was to be called in to see her.

To all medication she was extremely susceptible; even a few drops of dilute phosphoric acid seemed to do injury. Similarly tonics and stimulants were badly borne. Some mental depression accompanied all these symptoms; but the patient had considerable *force of will*, and when in good health was very energetic. There were no fits of laughing or crying.

After a careful and thorough trial of general faradization, central galvanization, and galvanization of the cervical sympathetic, I gave up the case. She bore electricity as she bore everything else—badly, and no amount of treatment succeeded in bringing her to that condition where she could tolerate an average dose of either treatment.

During the latter part of the treatment the uterus was carefully examined by Dr. Skene, who found a tendency to vaginismus and anteflexion; but these symptoms were regarded merely as accompanying or incidental phenomena, and were not treated.

After electrical treatment was abandoned time came slowly to the rescue, and, under the care of her physician, she so far improved as to be able to walk out, but she subsequently relapsed.

This case illustrates: First, that there are certain temperaments that will not bear electricity; secondly, that in nerve functional disturbances time, rest, and hygiene may cure or greatly relieve, after medication has failed. In all these cases special pains must be taken to avoid exertion, mental or mus-

cular. A slight indiscretion may put back the patient for weeks or months. For the nervously exhausted to overdo, even for an hour, is a blunder that is almost a crime.

From George Miller Beard, *Cases of Hysteria, Neurasthenia, Spinal Irritation, or Allied Affections* (Chicago: Spalding, 1874), 1–4, 6-7.

Auguste Tamburini
(1848–1919)
"A Theory of Hallucinations"
(1881)

The Italian Auguste Tamburini was an academic psychiatrist who served as both director of one of Europe's oldest asylums (San Lazzaro) and as a professor of psychiatry and neurology. He eventually became director of the psychiatric institute in Rome in 1907. Tamburini's article on the theory of hallucinations, published in French in 1881, was among the most cited in late nineteenth-century neuropsychiatry. It quickly inspired countless researchers and clinicians to see hallucinations no longer as possessing some kind of semantic meaning, but instead as products of mechanical processes involving the brain and nervous system.

There has been much debate on the nature and brain localization of hallucinations. This is to be expected as these phenomena are not only important symptoms of insanity and a cause of delusions, but have also played a role in history. Four types of explanation are available for their origin: (1) peripheral; (2) intellectual; (3) psycho-sensorial; and (4) sensorial.

According to the peripheral view, as stated in the writings of Erasmus Darwin, Foville and Michéa, hallucinations are subjective sensations caused by the peripheral irritation of sensory organs. As evidence, the authors quote the development of unilateral hallucinations in association with

morbid changes in peripheral sense organs or the distortion of image caused by manual pressure on the eye. This view fails to explain the presence of hallucination in cases when the peripheral sense organ has been completely destroyed.

The "intellectualistic" view of hallucinations attributes the origin to disturbances of imagination and memory and has been supported by Esquirol, Lauret, Lélut, Falret, Reil, Neumann, Parchappe, Brierre de Briesmont, Delasiauve, Maudsley, etc. According to these authors hallucinations are thoughts changed into sensations; they are projections, so to speak, of the sensorial aspects of ideas on to the external world. Hallucinations are sensory delusions. Against this view is the fact that hallucinations are often thematically unrelated to normal or pathological ideas. Likewise this hypothesis is unable to explain the clinical facts listed by those supporting the peripheral view.

The psycho-sensorial theory is a combination of the two views above, and explains more clinical observations than either of them alone. Both intellectual and peripheral centres would participate in the constitution of the hallucinatory phenomenon. The theory has been supported by Müller, Griesinger, Baillarger, Moreau de Tours, Marcé, Motet, and more recently by Ball. The problem with this view is that, because it describes hallucinations in a general way, it explains everything; even worse, it does not take into consideration recent advances in neurophysiology.

According to the fourth view, the one more in keeping with modern brain anatomy and physiology, hallucinations are the result of activity in the sensory centres of the brain. This view, therefore, postulates the existence of sensory centres in the central nervous system to which sensory information is conveyed by sensory nerves. Morbid changes in these brain sites would give rise to hallucinatory experiences. Already hinted at in the work of Baillarger and Schröder Van der Kolk, this view has recently been expressed with great clarity by Kahlbaum and Hagen, and adopted by Koppe, Jolly, Hoffmann, Luys and Ritti.

How is that sensations generated in these brain centres can give rise to images endowed with all features of reality? According to Hagen all peripheral stimuli arriving at the sensory centres are immediately diverted to two destinations: the ideational centres (where they will generate images in consciousness) and back to the periphery (by the principle of external projection). Stimuli generated in the brain sites themselves would suffer the

same fate, thereby giving rise to apparent perceptions. This view explains all clinical facts but requires periodical updating according to the progress of brain anatomy and physiology.

The next question is, where are the brain centres for hallucinations? Krafft-Ebing, Hoffmann, Leidersdorf and others have not provided an answer and simply refer to them as the sites where sensory nerves terminate. Hagen and Kahlbaum, on the other hand, have suggested that these centres are, in fact, the basal ganglia or sensory brain (*Sinnbirn*). Bergmann, in turn, has claimed that the sensory nerves terminate in the walls of the cerebral ventricles where, by means of a resonance mechanism, perceptions are constituted. Hallucinations would result from an irritation in these areas: the third ventricle would be associated with visual hallucinations, the fourth ventricle with auditory ones. . . .

Recent work, however, seems to suggest that sensory fibres do terminate in the cortex itself. . . . Panizza's "Observations on the optic nerve" was originally published in the Journal of the Lombard Institute for August 1855 and reprinted a year later. It reports experiments carried out on mammals, birds and fish by two methods: (1) selective lesioning of cortical sites followed by evaluation of visual function, and (2) eyeball enuncleation followed by analysis of retrograde nerve atrophy. Panizza wrote that in the dog "removal of cortical substance under the parietal areas causes contralateral blindness." He also showed retrograde atrophy of the optic nerve in the rabbit, horse, dog, ox, and sheep. He mapped the atrophic changes in the geniculate bodies, thalamus and even in the fibres arriving in the striatal area.

Clinical data suggest that this also applies to the human. For example, a traumatic lesion of the left eye caused atrophy in a three-year-old; a post mortem after his early death at age 16 showed parietal-occipital atrophy involving the right thalamus and hemisphere. After a right stroke a second patient developed amaurosis of the right eye and post-mortem showed softening of the posterior hemisphere. Panizza concluded that in the mammal the optic nerve receives contributions from the geniculate bodies, thalamus, and occipital lobe. His work has been replicated by Hitzig and Ferrier. . . .

These findings may help to explain the genesis of hallucinations. If there are cortical centres where sensory impressions become perceptions (and where they are also stored as mnemonic images), then it would be surprising that they did not play a role in the production of hallucinations. In the same way that irritation of motor centres may cause disorganized

(epileptic) movements, morbid changes in sensory ones would cause pathological sensations.

What, however, would the origin of the sensations be? They are likely to be mnemonic images, stored impressions, which after being revived by the morbid process would with varying vividness (according to the strength of the irritation) present themselves to consciousness. When accompanied by all sensory features they would be perceived as real and hence constitute hallucinations proper. When the irritation is unilateral or limited to a discrete cortical area, the hallucination will occur in one specific sense modality (e.g. visual or auditory) and be unilateral; if the morbid process is bilateral, diffuse and involving more than one sensory centre, a composite hallucination involving all sense modalities might be experienced.

Acceptance of this hypothesis should depend on its being (a) in keeping with what is known in brain physiology, (b) in agreement with the facts of clinical observation, and (c) able to explain the diverse clinical presentations of hallucinations as reported by those who postulate rival theories (peripheral, psychical or mixed).

With regards to (a) above it has already been stated that the sensory centre hypothesis is in keeping with current knowledge of cortical physiology. An inkling of this view, which would make sense even if no facts on cortical physiology were yet known (as it is presented in the work of Meynert, Wundt and Hughlings-Jackson), can be found in the work of Ferrier when he reported that eyeball and ear movements followed electrical stimulation of the sensory centres, and that these were likely to be related to the projection of experienced images on to the external world (which the experimental animal would perceive as real objects). Thus, whilst stimuli applied to the motor centres cause epileptic movements, stimuli applied to the sensory centres generate hallucinations.

But the hypothesis must also be in accord with clinico-pathological observations. The point here is to determine whether subjects presenting with clear cut hallucinations will also exhibit relevant cortical lesions on post-mortem. It must be remembered that hallucinations are a transient phenomenon which tends to occur during the early stages of psychoses and that the irritation that causes them in the first place is unlikely to persist until the patient comes to post-mortem. Hallucinations may also be replaced by the symptoms of mental degeneration. Furthermore, even if the subject died whilst experiencing hallucinations, lesions may not be visible

because of their irritative nature, as it is indeed the case with regards to irritative lesions in motor areas. Therefore, it is easy to imagine how difficult it is to accumulate evidence to confirm the hypothesis. Very few cases meeting these requirements have been reported in the literature (such as those by Ferrier, Pooley, Atkins and Gowers). Their common feature is that visual hallucinations were experienced only during the early irritative stage, and that the destructive stage of the lesion was followed by loss of vision. . . .

The third condition dictates that the hypothesis provide adequate explanation for all clinical presentations of hallucinations. For example, how to explain the fact that hallucinations precede or follow delusions. Both situations can be accommodated by this theory on the basis that it is unlikely that lesions causing hallucinations always occur on their own. Often, in fact, they are accompanied by other brain lesions. So, if the first lesion is on a sensory centre, hallucinations will precede delusions; if it is on the ideational centre it will be the other way around. This explanation is valid provided that it is not assumed that ideation is but a complex form of activity taking place in the sensory centres themselves.

In favor of a separation between ideational and sensory centres is the fact that psychologically healthy subjects have no difficulty in recognizing interloping sensations as hallucinations. This capacity for insight, however, cannot be explained either by the peripheral or psychical theory of hallucinations. The peripheral theory is also rendered implausible by the fact that hallucinations tend to be cognitively complex; and the psychical theory by the fact that they are often sensorially vivid. Both objections are easily dealt with by the theory that hallucinations result from irritation of the cortical sensory centres. . . .

It can be concluded that many aspects of hallucinations not explained by other theories are accounted for by the view proposed in this paper. This is based on the most recent anatomical, physiological and clinical findings, and postulates that the fundamental mechanism for hallucinations is a state of morbid excitation of cortical sensory centres, of sites where sensory impression from the peripheral organs are collected and transformed into perceptions, and where their mnemonic images are stored.

From Auguste Tamburini, "A Theory of Hallucinations," *History of Psychiatry* 1 (1990): 151–156 (© History of Psychiatry, 1990), by permission of Sage Publications Ltd.

Richard von Krafft-Ebing
(1840–1902)

Psychopathia Sexualis
(1892)

Since ancient times, Western religions have inveighed against various forms of deviant sexual behavior. And while medicine often appeared to be less prudish about commending the benefits of an active sex life, physicians nonetheless often warned of the health dangers posed by venereal diseases and masturbation. Over the course of the nineteenth century, however, a new trend emerged, as researchers and practitioners began applying clinical notions of normality and abnormality to classify, diagnose, and treat sexual deviants.

Richard von Krafft-Ebing's *Psychopathia Sexualis* was the culmination of this trend. An older contemporary of Emil Kraepelin and Sigmund Freud, Krafft-Ebing practiced psychiatry in various German asylums and clinics, until eventually settling in Vienna. *Psychopathia Sexualis* detailed his work with patients complaining of "perversions," which at that time included homosexuality, sadism, masochism, and exhibitionism. First published in German in 1886, the book was a scholarly best seller, going through numerous reprints and new editions during Krafft-Ebing's lifetime. As these excerpts show, the book was readily available to the general public, leading many readers to see their sexual proclivities in a new light. In fact, many readers were so moved that they contacted Krafft-Ebing and shared their own stories with him. He, in turn, incorporated many of their tales into subsequent editions of his book (see cases 136 and 137, below). As a result, *Psychopathia Sexualis* gives us both a sense of how Krafft-Ebing operated—note his reliance on hypnosis, a very common form of treatment at this time—as well as insight into how self-identified "perverts" lived with their desires.

While up to this time contrary sexual instinct has had but an anthropological, clinical, and forensic interest for science, now, as a result of the latest investigations, there is some thought of therapy in this incurable condition, which so heavily burdens its victims, socially, morally, and mentally.

A preparatory step for the application of therapeutic measures is the exact differentiation of the acquired from the congenital cases; and among the latter, again, the assignment of the concrete case to its proper position in the categories that have been established empirically.

The diagnostic differentiation of the acquired from the congenital condition is made without difficulty in the early stages of the anomaly.

If sexual inversion has already taken place, then the history of the development of the case will throw light upon it.

The important decision, prognostically, as to whether the contrary sexual instinct is congenital or acquired, can only be made in such cases by means of the most minute details of the history.

The establishment of the fact that contrary sexual instinct existed before indulgence in masturbation is of great importance with reference to deciding whether the anomaly is congenital or not. In this, however, a difficulty arises, owing to the possibility of imperfect localization of past events (illusions of memory).

For the presumption of acquired contrary sexual instinct, it is important to prove the existence of hetero-sexual instinct before the beginning of solitary or mutual onanism.

In general, the acquired cases are characterized in that:—

1. The homo-sexual instinct appears secondarily, and always may be referred to influences (masturbatic neurasthenia, mental) which disturbed normal sexual satisfaction. It is, however, probable that here, in spite of the powerful sensual libido, the feeling and inclination for the opposite sex are weak *ab origine*, especially in a spiritual and aesthetic sense.
2. The homo-sexual instinct, as long as inversio sexualis has not taken place, is looked upon, by the individual affected, as vicious and abnormal, and yielded to only *faute de mieux*.
3. The hetero-sexual instinct long remains predominant, and the impossibility of its satisfaction gives pain. It weakens in proportion as the homo-sexual feeling gains in strength.

On the other hand, in congenital cases (a) the homo-sexual instinct is the one that occurs primarily, and becomes dominant in the vita sexualis. It appears as the natural manner of satisfaction, and also dominates the dream-life of the individual. (b) The hetero-sexual instinct fails completely, or, if it should make its appearance during the life of the individual (psycho-sexual hermaphroditism), it is still but an episodical phenomenon which has no root in the mental constitution of the individual and is essentially but a means of satisfaction of sexual desire. . . .

The prognosis of the cases of acquired contrary sexual instinct is, at all events, much more favorable than that of the congenital cases. In the former, the occurrence of effemination—the mental inversion of the individual, in the sense of perverse sexual feeling—is the limit beyond which there is no longer hope of benefit from therapy. In the congenital cases, the various categories established in this book form as many stages of psycho-sexual taint, and benefit is *probable* only within the category of the psychical hermaphrodites, though *possible* (*vide* the case of Schrenk-Notzing) in that of the urnings.*

The prophylaxis of these conditions becomes thus the more important,—for the congenital cases, prohibition of the reproduction of such unfortunates; for the acquired cases, protection from the injurious influences which experience teaches may lead to the fatal inversion of the sexual instinct.

Numerous predisposed individuals meet this sad fate, because parents and teachers have no suspicion of the danger which masturbation brings in its train to such children.

In many schools and academies masturbation and vice are actually cultivated. At present much too little attention is given to the mental and moral peculiarities of the pupils. If only the tasks are done, nothing more is asked. That many pupils are thus ruined in body and soul is never considered. In obedience to affected prudery, the vita sexualis is veiled from the developing youth, and the slightest attention given to the excitations of his sexual instinct. How few family physicians are ever called in, during the years of development of children, to give advice to their patients that are often so greatly predisposed!

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*Editor's note: *Urning* was a nineteenth-century term for "homosexual."

It is thought that all must be left to Nature; in the meantime, Nature rises in her power, and leads the helpless, unprotected innocent into dangerous by-paths.

A more detailed treatment of this prophylactic side of the subject is impossible here.

To parents and teachers, the experiences detailed in this work, and numerous scientific works on masturbation, give suggestions.

The lines of treatment, when contrary sexual instinct exists, are the following:—

1. Prevention of onanism, and removal of other influences injurious to the vita sexualis.
2. Cure of the neurosis (neurasthenia sexualis and universalis) arising out of the unhygienic conditions of the vita sexualis.
3. Mental treatment, in the sense of combating homosexual, and encouraging hetero-sexual, feelings and impulses. . . .

Case 136. *Acquired Contrary Sexual Instinct.*— Mr. Z., aged 32, divorced. He comes of a hysteropathic mother. Maternal grandmother suffered with hysteria, and her brothers and sisters were neurotic. One brother is an urning. Z. was but poorly endowed mentally, and did not learn easily. No sickness besides scarlatina. When thirteen, he was taught to masturbate by companions in a school. Sexually, he was hyperaesthetic, and, at seventeen, began to indulge in coitus, with full pleasure and power. For reasons of position and money, he married at twenty-six. The marriage was very unhappy. After a year Mrs. Z. became incapable of coitus, by reason of uterine disease. Z. satisfied his inordinate desires with other women, *faute de mieux*, by masturbation. Besides, he gave himself up to play, led an absolutely dissolute life, became exceedingly neurasthenic, and sought to strengthen his weakened nerves by drinking great quantities of wine and brandy. To his essential cerebral asthenia were added peripheral alcoholic cramps and globus, and he became very emotional. His libido nimia continued unabated. On account of his disgust of prostitutes and fear of infection, satisfaction by coitus was exceptional. For the most part, the patient helped himself with onanism.

Four years ago he noticed weakening of erection and decrease of libido for women. He began to feel himself drawn toward men, and his lascivious dreams were no longer concerned with women, but with men.

Three years ago, while being rubbed by a bath-attendant, he became powerfully excited sexually (the attendant also had an erection, to patient's surprise). He could not keep from embracing and kissing the attendant, and allowing him to perform masturbation on him, the attendant doing it most willingly. From this time this mode of indulgence was all that he cared for. Women became a matter of entire indifference to him; he devoted himself exclusively to men. With them he practiced mutual masturbation, and had a longing to sleep with them. He abhorred pederasty. He was entirely satisfied until (August, 1890) an anonymous letter, warning him to be careful, brought him to his senses. He was much frightened, had hysterical attacks, and became much depressed. He was embarrassed before men, seemed like a pariah in society, contemplated suicide, and finally confessed to a priest, who comforted him. He now fell into a religious state (equivalent), and, out of remorse and to cure himself of his abnormal sexual inclinations, wished to go into a cloister. While in this state, my "Psychopathia Sexualis" fell into his hands. He was frightened and filled with shame, but found comfort in it, inasmuch as he concluded that he must have some malady. His first thought was to rehabilitate himself sexually in his own eyes. He overcame all disinclinations, and visited a brothel. At first he was not successful, on account of great excitement, but he finally succeeded.

Since, however, his contrary sexual inclinations were not overcome, in spite of all his efforts to put them down, he finally came to me, asking for assistance. He felt himself to be terribly unfortunate, and very near to despair and suicide. He saw destruction before him, and would be saved at any price.

His confession was interrupted by numerous hysterical attacks. Comforting and encouraging words about his future had a calming influence. . . .

Hip-baths, massage, ergot with antipyrin and pot. brom., ordered, with interdiction of onanism, intercourse with men, and lascivious thoughts of them.

After a few days the patient came complaining that he was not equal to the task. He said his will was too weak. In this precarious situation, it seemed that nothing but hypnotic treatment could bring improvement.

September 11, 1889. First Sitting. Bernheim's method [of hypnosis] used, in order to induce lethargy as quickly as possible.

Suggestions:—

1. I abhor onanism, and will not masturbate again.
2. I regard the inclinations for men disgusting,—horrible; and I shall never think men handsome and enticing.
3. Women alone I find enticing. Once a week I shall cohabit with pleasure and power.

The patient received these suggestions, and repeated them in a drawling tone.

The sittings took place every second day. After the fifteenth, it was possible to induce the somnambulistic stage of hypnosis with any post-hypnotic suggestions desired.

The patient improved morally and mentally, but symptoms of cerebral neurasthenia troubled him still, and, now and then, dreams of men occurred; and there were, also, in the waking state, inclinations towards men, which depressed him exceedingly. . . .

December 9, 1889, patient again came for treatment. Of late he had had lascivious dreams of men twice, but had experienced no inclination toward men in the waking state. He had also resisted the impulse to masturbate, though, while living alone in the country, he had had no opportunity for coitus. He had inclinations only for the opposite sex, and, as a rule, dreamed only of females. Returned to the city, he had indulged in coitus with pleasure. The patient felt himself morally rehabilitated, being almost free from neurasthenic symptoms; and, after three more hypnotic sittings, he declared himself perfectly well, and confident that he would not relapse. Such a relapse occurred, however, in September, 1890, when, after over-exertion on an excursion into the mountains, and emotional strain with want of opportunity for coitus, he again became neurasthenic.

Again he had dreams of men, and felt drawn toward attractive male forms; he masturbated many times, and, after returning to the city, found no real pleasure in coitus. By means of anti-neurasthenic treatment and hypnosis, it was possible to restore the previous condition.

In the course of the years 1890 and 1891 the patient now and then had contrary sexual feelings and dreams, but only when, as a result of emotional strain or excesses, his neurosis re-appeared. At such time satisfaction in coitus was wanting. He would then find it necessary to undergo a few hypnotic sittings, in order to restore his equilibrium—always with success.

At the end of 1891 the patient pointed with satisfaction to the fact that, since treatment, he had been able to avoid masturbation and male-intercourse, and had regained his self-confidence and self-respect.

Case 137. "I was born in 1858, out of wedlock. It was only late that I was able to trace my obscure origin, and obtain knowledge of my parents; and this knowledge is, unfortunately, very obscure and imperfect. My father and mother were cousins. My father died three years ago. He later married, and, as far as I know, had several healthy children.

"I do not think that my father had contrary sexual feelings. Without knowing him as my father, I often saw him when I was a child. He was a powerful, masculine man. As for the rest, it is said that, at the time of my birth, or before, he was sexually ill.

" . . . I think I may say that my vita sexualis was really first awakened after I had been seduced into mutual masturbation, in my thirteenth year, by a room-mate at the Institute. At that time ejaculation did not take place, but first about a year later. Nevertheless, I gave myself up to the vice of onanism passionately. At this time, however, the first signs of homo-sexual inclination were manifested. Youthful, powerful men, market-helpers, workmen, and soldiers took possession of my dreams and played an important role in my fancy while masturbating.

" . . . When, at the age of fourteen, I went to H., I lost sight of my lover and seducer. He was some years older than I, and was an official; and, in this capacity, when I was nineteen, I again met him once on the railway. We immediately cut the journey short, and lodged together, attempting mutual pederasty; but, on account of pain, immissio was not successful. We amused ourselves in mutual onanism. In H. I had sexual intercourse with two fellow-students, but this intercourse was confined to frequent mutual onanism, owing to the fact that they were not inclined to pederasty. . . .

"With my sojourn here, my vita sexualis has undergone a complete change. I have learned how easy it is to find persons who, partly for money and partly for desire, yield to our inclinations. I have also not been spared annoying experiences with cheats. Until the end of the last year (since then, owing to fear of venereal infection, I have not gone beyond mutual masturbation), I enjoyed male-love to the full extent, particularly in passive pederasty. I have never practiced active pederasty, because I have found no one able to endure the pain.

"Generally, I seek my lovers among cavalrymen and sailors, and eventually, among workmen, especially butchers and smiths. Robust forms, with healthy facial complexions, attract me especially. Leathern riding-trousers have a particular charm for me. I have no partiality for kissing and the like. I also love large, hard, and calloused hands.

"I do not wish to leave unmentioned that, under certain circumstances, I have great control of myself.

" . . . Until my thirty-eighth year I had not a clear understanding of my condition. I always thought that, by early and frequent masturbation, I had become averse to women, and hoped always that, when the right woman came, I should be able to abandon onanism and find pleasure in her. Here it was that I first came to fully understand my condition, after making the acquaintance of others suffering and feeling like myself. At first I was frightened; later I came to look upon my fate as something not dependent on myself. Too, I made no further effort to resist temptation.

"Two or three weeks ago 'Psychopathia Sexualis' fell into my hands. The work has made an unexpectedly deep impression on me. At first I read the work with an interest that was undoubtedly lascivious. The description of the cultivation of *mujerados*, for example, excited me uncommonly.* The thought of a young, powerful man being emasculated in this manner, in order, later, to be used for pederasty by a whole tribe of wild, powerful, and sensual Indians, so excited me that I masturbated five times during the next two days, fancying myself such a presumptive *mujerado*. The farther I read in the book, however, the more I saw its moral earnestness; the more I felt disgust with my condition; and the more I saw that I must do everything,

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*Editor's note: George Miller Beard, for instance, describes *mujerados* as a group of men among the Pueblo Indians with "protuberant abdomens, well-developed mammary glands, rounded and soft limbs, shrunken genital organs, high, thin, cracked voices, and pubes devoid of hair. . . . In order to make a Mujerado a very strong man is selected; masturbation is performed upon him many times a day; he has to ride almost continuously on horseback without saddle. By this process the genital organs become much excited, and seminal losses are produced; the nutrition of the organs is interfered with; they grow smaller and weaker, and, in time, desire and power cease; then follow the changes in character, the desire to dress like a woman and to engage in feminine occupations, just as with the Scythians; courage and manhood are lost; wives and children, for those who have them, pass from their control." George M. Beard, *Sexual Neurasthenia (Nervous Exhaustion): Its Hygiene, Causes, Symptoms, and Treatment, with a Chapter on Diet for the Nervous* (New York: E. B. Treat, 1886), 100–101.

if it were possible, to bring about a change in my condition. When I had finished the book, I was determined to seek assistance from its author.

"The reading of this work had an undoubted effect. Since then I have masturbated only twice, and have practiced onanism with cavalrymen only twice. In every instance I have had really less pleasure and satisfaction than before, and I always have the feeling: 'Ah, if I could only be free from it!' Nevertheless, I confess that, even now, in the society of handsome soldiers, I immediately have erection.

"In conclusion, I may add that, in spite of, or, perhaps, on account of, onanism, I have never had pollutions. The ejaculation of semen, which usually consists of only a few drops, and it has always been so, takes place only after prolonged friction. If, for any reason, I have not masturbated for a long time, the ejaculation takes place quickly, and is more abundant. About twelve years ago Hansen tried in vain to hypnotize me." *

In the spring of 1891 the writer of the foregoing autobiography visited me, with the declaration that he could live no longer in his condition; that he looked to hypnotic treatment as the only hope of salvation, for he had not strength enough to resist his impulse to masturbation and satisfaction with persons of his own sex. He felt like a pariah; like an unnatural man; like one outside the laws of nature and society, and in danger of criminal prosecution. He felt moral repugnance when he performed the act with a man, but yet the sight of any handsome soldier actually electrified him. For years he had not had the slightest sympathy with women, not even mentally.

The patient looked to be exactly the person, physically and mentally, described by himself in his autobiography. His head was exquisitely hydrocephalic, and also plagiocephalic. At first attempts at hypnosis met with difficulties. Only by Braid's method, with the help of a little chloroform, was deep lethargy attained at the third sitting.† From that time simply looking at a shining object was sufficient. The suggestions consisted of the command to avoid masturbation, the removal of homosexual feelings, and the

*Editor's note: He is referring here to Carl Hansen (1833–1897), a Danish hypnotist who performed on stage in Vienna around this time.

†Editor's note: Braid's method refers to the method of hypnosis developed by James Braid (1795–1860), the Scottish surgeon who first coined the term *hypnosis*. Braid emphasized the importance of relaxation, directed attention, and suggestion in a successful hypnosis.

assurance that the patient would have inclination for women and be virile, and have pleasure only in hetero-sexual intercourse. Masturbation was indulged in but once; after the eighth sitting the patient dreamed of a woman.

When, after the fourteenth sitting, the patient had to return, on account of pressing business, he declared that he was quite free from any inclination to masturbate and to indulge in male-love, but that he was by no means absolutely free from his partiality for men. He felt returning interest in the female sex, and hoped to be freed finally from his unhappy condition by continuance of the treatment.

From Richard von Krafft-Ebing, *Psychopathia Sexualis* (Philadelphia: F. A. Davis, 1892), 319–321, 330–308.

Jean-Martin Charcot

(1825–1893)

"A Tuesday Lesson: Hysteroepilepsy"

(1888)

During the last quarter of the nineteenth century, the French neurologist Jean-Martin Charcot was among the most acclaimed experts in the world on hysteria. Conducting research and teaching at the famous Salpêtrière Hospital in Paris (1862–1893), Charcot used a novel combination of clinical observation, pathological anatomy, and photography to analyze hysterical episodes, breaking them down into discrete stages in order to enable more accurate diagnosis. By 1883, Charcot had turned the Salpêtrière into a hub for the study of hysteria: of the some five hundred women admitted to the hospital at that time, around 20 percent were diagnosed with hysterical symptoms. Charcot treated these mostly working-class patients with a regimen that included the use of ovarian compressor belts, hypnotism, and electrotherapy.

Charcot was a renowned self-promoter. His fame grew especially after he introduced a set of weekly public lectures, attended by

students, physicians, researchers, artists, and writers from all over Europe. On Fridays, he gave carefully prepared presentations, employing diagrams, drawings, art, and even costumed patients to make his points. The Tuesday Lessons were reserved for more impromptu discussion of cases and for interviews with newly admitted patients. In both settings, Charcot typically illustrated his theses by using hypnotism to induce hysterical symptoms in his patients. The dramatic features of the lectures did not escape the notice of contemporaries, and, as the excerpt below shows, critics questioned whether Charcot's patients were merely getting caught up in the medical theater being staged.

(A female patient on a stretcher is brought into the amphitheater.)

CHARCOT: Here is a patient whom you saw last Friday. After a fall she developed a lower extremity contracture with a deformity of her right foot. Nothing is more frequent in hysterics than posttraumatic contractures. What could one make of such a case? I told you last time how important it is to treat and cure these contractures as soon as they appear. But now here we have an exception to this rule, and we have waited and watched this woman three or four days without interfering. I told you why we did this—with cases like this woman's, you may, in fact, be able to treat this through provoking a second sort of attack. Often with such attacks, a change occurs in the patient and a contracture that seemed permanently fixed before can completely disappear. You may say to me, "Isn't there something immoral about waiting and provoking such crises?" Surely not, if one can offer a treatment for a disorder that otherwise has no cure.

And I have shown you how there is a parallel relationship between transient hysteric attacks and the forms of hysteria like this one that last longer, five or six months. Often, those patients with contractures are not those who have fleeting hysteric attacks and vice versa. It is because of this doctrine, so soundly described by Dr. Pitres, that we can make use of hysterogenic points to provoke a transient attack as a form of therapy in the treatment of static hysteric signs. Now, this patient will be used for demonstration. I will tell you, however, that although I am practically certain of the outcome of this experiment, man is less predictable than machinery, and I will not be totally surprised if, in fact, we do not succeed. I have also

A Clinical Lesson with Charcot at the Salpêtrière 1887, by Andre Brouillet (1857–1914). This famous portrait depicts the neurologist Jean-Martin Charcot during one of his public lectures. Here he discusses the case of a patient (Blanche Wittmann), who has just been hypnotized by his assistant Joseph Babinski. Hopital Neurologique, Lyon, France. © Erich Lessing/Art Resource, NY.

heard that animal experiments performed before an audience often given different results from those seen in the laboratory. This may be the case here, since this is, in fact, a comparable clinical experiment. If we do not get the desired result, it still will be a significant lesson for you.

This patient has a hysterogenic point on her back, another under her left breast, and a third on her leg. We will focus on this latter one. If the attack proceeds as I believe it will, I will want you to focus on all its phases. This is not an easy task, and it took me many years to analyze the phenomena you will see. I first came to the Salpêtrière 15 or 20 years ago and inherited the well-run service of Dr. Delasiauve. From my first days I witnessed these hysteroepileptic attacks, and was very circumspect in making my early diagnoses. I said to myself, "How can it be that such events are not described in the textbooks? How should I go about describing these events from my first-hand experience?" I was befuddled as I looked at such patients, and this impotence greatly irritated me. Then one day, when reflecting over all these patients as a group, I was struck with a sort of intuition about

them. I again said to myself, "Something about them makes them all the same." Indeed we have a particular disease before us—primary hysteria beginning with an epileptiklike attack that resembles so closely real epilepsy that it may be called hysteroepilepsy, even though it has nothing at all to do with true epilepsy. The epileptoid phase can be divided into a tonic and clonic portion. Then, after a brief respite, the phase of exotic movements begins, under one or two predominant forms, either vocalizations or extreme opisthotonus (*arc en cercle*). Then, the third phase supercedes, and suddenly the patient looks ahead at an imaginary image—indeed a hallucination, which will vary according to the setting. The patient may look with great fear or with joy, depending on what she sees. You saw this in a woman the other day when I touched her abdomen in the ovarian region. She rose from her bed, hurried into the corner, and said the most distressing things.

But I want you to appreciate especially the unfolding of an attack. I tell you all this beforehand so that you can mark each phase, since they are hard to appreciate without preparation. Importantly, the attack is not a series of individual small attacks, but a single event that unrolls sequentially. I use here the method of describing an archetype with the most complex and fully developed features described. This system is essential for all neurologic diagnosis; one must learn to identify the archetype. The epileptoid phase can be lacking and the attack begin with the movement phase, either vocalizations or back arching. Sometimes the movements never appear, and one only has hallucinations. There are as many as 20 variations, but if you have the key to the archetype, you immediately focus on the disease at hand and can say with confidence that in spite of the many possible variations, all these cases represent the same disorder. So, here we have this contracted foot that reportedly cannot be reduced either during the day or night. I have not specifically examined it at all times, but I surmise that this is in fact true. We are not dealing here with simulation, one of the greatest obstacles to neurology. *(The intern touches the hysterogenic point under the left breast. Immediately, the attack begins).*

CHARCOT: Now, here we have the epileptoid phase. Remember this sequence—epileptoid phase, arched back, then vocalizations. The arched back that you now see is rather pronounced. Now here comes the phase of emotional outbursts, which fuses with the back arching, and now there is a contracture phase. Such contractures can persist occasionally, and if this

occurs in our patient, we will hardly have helped her. Now the epileptoid period starts again. Focus your attention this time on the two distinct epileptoid movement phases—first, the tonic, then the clonic. Note how this resembles true epilepsy. Now let us see if she is ovarian. *(The intern comes forwards and presses the ovarian region).*

CHARCOT: Do this in a real epileptic and nothing will happen, showing you immediately the difference between epilepsy and hysteroepilepsy. In contrast to this situation, epilepsy has no direct link with the ovary. See how the attack is momentarily suspended by abdominal compression. Is it true that ovarian compression actually aborts the attack? This maneuver is contested in a number of textbooks, where the authors act as if they know what they are talking about. In both England and Germany there are some people who say they have never seen ovarian compression work, but these same people are those who are all too eager to generalize from their very limited experience. In that the phenomenon has been unequivocally demonstrated to occur in Paris, I find it only reasonable to believe it also occurs elsewhere.

Now we will release the compression, and you will see how the attack promptly recommences. Here comes the epileptoid phase again. Often outside of France, epileptoid behavior is still called epilepsy. I disagree with such terminology and distinctly calls this hysteroepilepsy or hysteria major. Here now comes the arched back. Note the consistent pattern, always predictable and regular. . . .

Let us press again on the hysterogenic point. Here we go again. Occasionally subjects even bite their tongues, but this would be rare. Look at the arched back, which is so well described in the textbooks.

PATIENT: Mother, I am frightened.

CHARCOT: Note the emotional outburst. If we let things go unabated, we will soon return to the epileptoid behavior. Now we have a bit of tranquility, or resolution, followed by a type of static contracted posture. I consider this latter deformity as an accessory phenomenon to the basic attack. *(The patient cries again: "Oh! Mother.")*

CHARCOT: Again, note these screams. You could say it is a lot of noise over nothing. True epilepsy is much more serious and also much more quiet.

I do not know what will be the final outcome for this woman's contractures, but I am glad to have been able to show you a rather typical

attack. Let us review for emphasis: an epileptoid phase with two parts, tonic and iconic, followed by a phase of exotic movements, and then a phase of high emotional pitch, which, in this patient, is sad. All these are then followed by these strange contorted postures.

This patient was just a minute ago quite stiff, which is unusual. Most patients look quite natural and assume realistic poses during all of this. A final phase is quite rare and not seen in this patient—a period of delirium.

Here, after the phase of affective change, the cycle starts over and may continue for several days. Ovarian compression is effective only for some patients—these are called ovarian subjects. Clearly, all subjects are not ovarian. I can only emphasize again my stand on this, even though others have misquoted my opinions. From such misquotes, I have been said to advocate surgical operations in the form of ovarian ablation for hysteroepilepsy, such as are performed in America. What I have said is still true; there are certain patients who have ovarian tenderness and in such patients, ovarian compression can stop an individual attack, although not the disease. When I say stop an attack, I mean you can provide your patient with a respite. We will place around this patient's abdomen a compression belt, and she will temporarily be controlled. But some day she will have to remove it—she can't wear it forever, and she may well start her spells all over again. Ovarian compression is a preventative method and also a means to assure temporary peace. But I emphasize again, it is not a cure. Nor is ovarian resection. Do not be fooled, the ovary is not the only spot that can be compressed for effective control. . . .

It would seem by some accounts that hysteroepilepsy exists only in France; in fact, it has been said that it exists only at the Salpêtrière, as if I have created this condition by my own willpower. What a marvel this would be if I could, in fact, fabricate illnesses according to my whims or fantasies. But in fact all I am is a photographer. I describe what I see. And it is all too easy to show you that such phenomena have indeed occurred outside the walls of the Salpêtrière. First, the descriptions of possessed victims from the Middle Ages are full of similar descriptions. Dr. Richer,* in his monograph, showed how in the fifteenth century, the same syndrome

*Translator's note: Paul Richer was both artist and neurologist and always a close collaborator of Charcot. The reference is to Richer's *Clinical Studies on Hysteroepilepsy* (with 105 drawings and 9 engravings) (Paris: Delahaye et LeCrosnier, 1881).

occurred just as it does today. Furthermore, I have received numerous contemporary personal reports, primarily from North American sources, that have no inherent relationship to the Salpêtrière. These letters were inspired by my reports of hysteroepilepsy and demonstrate that elsewhere cases exist that are exactly comparable to our cases here.

In England there is a highly distinguished physician, Dr. Gowers, who does not believe my descriptions. He sees things quite differently. In his treatise on epilepsy he uses the term "hysteroid conditions" after epilepsy. He considers the first "epileptic phase" that we have seen today as true epilepsy and agrees that all the subsequent phenomena you have witnessed occur, but he calls them postictal. Why? Because the patient's crisis always starts with what appears to be epileptic. We are seeing the same things and calling it by different names. I maintain that the sequence of events is a single process and is fixed in stepwise relationship of each phase. It forms an entity called hysteroepilepsy, and I will not be convinced otherwise.

Prior to my becoming director of this service, my predecessors introduced terminology to distinguish patients with these mixed attacks (*attaques à crises mixtes*) of hysteroepilepsy from those with distinct attacks of alternating hysteria and true epilepsy (*attaques à crises separées*). What does this latter term mean?

Let us take a look at another patient. (*Another woman comes in.*) From time to time, this woman has various attacks. She is hysterical but also truly epileptic. By this I mean she has two distinct and essentially different diseases, both belonging to the same general family as would be, for instance, gout and rheumatism occurring in the same patient. Now the two conditions are separate and remain so throughout the patient's life. They do not fuse or evolve one into the other. Let us not incorrectly create a Darwinism of such events. The pivotal feature in the doctrine of evolution is time, meaning multiple generations, and when I speak of two separate disorders, I speak of them in the context of one person's brief life.

In the case of separated crises, you have first an attack of hysteroepilepsy and then an attack of real epilepsy. In such a case, after a hysterical outburst, a patient may be found to have bitten her tongue. When the staff gets the patient back to bed and examines here, they will say, "No, she has had a real seizure" if she bit her tongue, and "Yes, it was a hysterical spell" if there was no tongue biting. The point here is that the two are entirely different types of events, although they may occur in the same patient. If one

reports that a patient had a real seizure, this behavior is taken seriously. If the same patient is said to have a hysterical fit, even lasting six days, there would be no major concern. If she has a real seizure, she could progress to status epilepticus, so the physician is immediately notified. The temperature could rise. Life itself would be at stake. Therefore, the distinction between the two is paramount. In hysteroepileptic patients with a known hysterogenic point, you could compress it, not only experimentally, to induce hysterical attacks, but more importantly, to stop them; whereas, if the attack is really a seizure, such compression will serve absolutely no purpose.

I will add that whereas potassium bromide has a palliative effect on true epilepsy, it will not help hysteroepileptic events. You can give tons of it without changing these patients. Primary hysteria is not epilepsy. Only in the patient's family tree will the two link together. By this I mean that a hysteroepileptic parent can have a child with true epilepsy and vice versa. But to be truthful, they could just as well give birth to manics or other children with forms of psychosis as well. I have told you before that the neurologic tree has many branches, and each one bears different fruits.

From Jean-Martin Charcot, *Charcot the Clinician: The Tuesday Lessons*, translated by Christopher G. Goetz (New York: Raven, 1987), 102–109. With permission of Christopher G. Goetz.

Emil Kraepelin

(1856–1926)

"About the Surveillance Ward at the Heidelberg Clinic for Lunatics"

(1895)

Emil Kraepelin's influence on modern psychiatry is difficult to overestimate. His insistence that psychiatric disorders be understood in terms of their symptoms and course (not their etiology), his effort to identify statistical regularities in symptoms, his use of experi-

mental techniques, and his development of a clinical picture for dementia praecox (schizophrenia) and cyclothymia (manic depression, or bipolar disorder) set in motion an agenda pursued by clinical psychiatry up to this very day. Assuming the position of director of the psychiatric clinic at the University of Heidelberg in 1891, he turned the facility into a site that combined teaching, research, and treatment.

The excerpt here describes his refinement of the surveillance ward at the Heidelberg clinic. Surveillance wards—large rooms that allowed staff to keep patients under "constant observation"—were first developed in Germany in the 1880s. These "scientific observatories," as one psychiatrist described them, constituted a new kind of institutional space, designed specifically to address the needs of a new urban, university-based psychiatry. Note the prevalence and use of psychoactive drugs on the ward.

I had been convinced of the need for a sufficiently large surveillance ward since my first year working at the Heidelberg clinic. Because of constant overcrowding in those rooms originally earmarked for the use of the surveillance ward, I had already considered it necessary to plan on a transfer from those wards to the adjoining and far more spacious ward for the semi-calm. With this revolution, I managed to completely get rid of the semi-calm ward during the day, in order to gain more space for scientific research rooms. Those rooms made available from the earlier surveillance ward were now only used as sleeping halls, whereas the small number of "semi-calm" patients were distributed among the three remaining wards—the surveillance ward and those for calm and agitated patients. . . .

At the outset, it must be granted that the described layout of the surveillance ward, as it once was, has certain significant disadvantages. It is certainly correct to insist that, above all else, a surveillance ward allow a clear view so that an unobstructed surveillance of all residents is possible. Admittedly in our clinic, rooms A–D are connected to one another via wide door openings, but it is not possible for a person to responsibly keep watch over more than at most two rooms at the same time. Fortunately such an awkward observation of each individual patient in the entire ward is not altogether necessary, as will become evident, so that practical operations have

not been hindered in any noticeable way by the inadequate layout of the ward. In fact, it has proven to be an estimable advantage that, in some directions, the peculiar arrangement of rooms allows for a more effective segregation on the surveillance ward of individual patients with different classifications.

In order to get an exact picture of all relevant circumstances [on the ward], special questionnaires were filled out for 250 days—from 3 January to 9 September 1893 (in addition to the standard daily reports following the Munich model). These contained, first of all, a record of the names of all the patients who found themselves on the surveillance ward from day to day, followed by a brief report on the apparent reason for their stay. Attention was also paid to those patients transferred to the surveillance ward only at night. In addition, the names of all patients in beds in other wards (for whatever reason) were also included. Furthermore, all tranquilizer and sleeping medication doses were continuously recorded, along with the particular reasons and the level of success. The duration of and reasons for every individual isolation were also entered. Finally, a survey of the distribution of patients in different wards during the day and at night was conducted on a regular basis. The 250 questionnaires for men and women that were collected were each put into 5 consecutive groupings, each covering 50 days, in order to be able to track changes in ward operations during the entire period of observation.

The daily average occupancy of the clinic during the entire time was 109.34 patients, of which 62.54 were in the male and 46.80 in the female ward. Of this total number, an average of 46.65 patients (42.66 percent) found themselves in both wards, 23.08 (36.89 percent) of the men and 23.57 (50.37 percent) of the women. The surveillance ward for men accommodated, then, over one-third of the total number, while the one for women an average of one-half. The difference derives in part from the larger frequency of depressive and agitated conditions among the women. Among the daily residents of the surveillance ward, depressed and agitated patients made up 12.04—around 19 percent—of the total number of male occupants, while for women the number was 15.48, i.e., no less than 33 percent of all female patients. In addition, it should naturally be noted that the spatial expansion of the surveillance ward made increasing the number of beds for more than 25 additional patients very difficult and, thus, the impact of the changing number of total residents was felt more on the remaining

wards than on the surveillance ward. During the entire observation period, the latter was filled almost to capacity on both sides of the clinic. The total amount of floor space in the patient rooms of the surveillance ward was around 231 square meters. Thus, on the basis of average occupancy, each patient had approximately 10 square meters of floor space or, since the height was 3.8 meters, 38 cubic meters of air space. . . .

The reasons why individual patients were placed in the surveillance ward, according to the survey report, were varied. In the case of the first group of patients, indications primarily involved somatic issues; the task of the surveillance ward was "care" for the patient in the stricter sense of the term. This includes those with genuine physical ailments of all kinds, namely those sick with fever. These are joined by the lame, paralytics in advanced stages, apoplectics, as well as those who are unhelpful and bedridden because of age, weakness, or other frailty or otherwise place greater demands on staff. Also the unhygienic should be counted here, especially those largely falling under the latter category of care. At any rate, there are also some feeble-minded, catatonics, and similar types who are not frail, but nevertheless, because of their lack of hygiene, require the better care provided by the surveillance ward. Finally, the physical condition of those who refuse food is a primary reason for accommodation on the surveillance ward. All patients who do not eat enough food are looked after in bed and are placed under constant supervision.

Those refusing food constitute the second main group of surveillance ward residents—those patients who suffer from severe depressive conditions. The primary task of the surveillance ward here consists of monitoring. It is well known that these patients provided the initial impetus for the construction of surveillance wards. All anxious and sad patients are part of this group, as long as there is concern about suicidal tendencies; also those patients who appear to be a danger to themselves for other reasons and, finally, those in a daze or stupor who do not fall under the first grouping, since one often has to reckon with sudden, unexpected, dangerous behavior on their part.

Besides monitoring, the surveillance ward offers these patients the possibility of lengthy bed rest, something that must be considered a form of therapy. This is perhaps the case to a greater extent for another group of patients whom we primarily try to treat on the surveillance ward: the frenetic. The majority of these are manic patients, especially periodic and

circular forms, as well as some paralytics and hebephrenics. These patients, like those who are anxiously agitated and demanding, can be supported most easily in bed on the surveillance ward and, in this manner, be satisfactorily influenced.

Alongside the three main groups of patients discussed so far, there is a small number of persons on the surveillance ward who are best accommodated here for a variety of reasons. The common goal is generally a more precise observation of the patients. Here it would be worth mentioning the new admissions about whom there are questions as to which of the above groups they might belong, prisoners under investigation, morphine and cocaine addicts, epileptics requiring special and on-going examinations (body weight, urine samples, digestion tests, etc), and, finally, some patients requiring a certain measure of segregation while being monitored because of particular sensitivities, deformities, or infectious or disgusting ailments. . . .

To get a full picture of operations on the surveillance ward, it is necessary to know the extent to which narcotics and sleeping medications have been given. If we count all those medications for which we methodically distribute a daily dose of an opium or bromine treatment, a daily average of 7.16 men and 7.36 women received tranquilizers or sleeping medications. Due to the varying occupancy levels on both wards, the percentage for men would be 11.45 percent and 15.74 percent for the women. The higher figure for women can be explained easily, being due to the fact that disproportionately more agitated patients were registered among their ranks. On average, among the men, 8.5 percent of day shift and 6.1 percent of night shift patients were counted as agitated; for the women, the figures were 24.4 percent for the day shift and 8.3 percent for the night shift. The large discrepancy during the day shift moved toward parity during the night shift, something that perhaps speaks to the fact that the agitation of women is determined to a greater degree by external influences than is the case with the men. Also the greater frequency of periodic manic and circular forms of milder agitation among the women vis a vis the more severe paralytic, epileptic, and alcoholic frenetic conditions found among the men plays a certain role here.

Considering the significantly higher number of agitated patients among the women, the amount of sleep medications and tranquilizers administered on the male side appears disproportionately too large. The cause of

this most probably lies in the exceptionally high number of patients. The male ward can normally accommodate around 55 patients; nevertheless, on average there were 62.5 patients here, and, one day, the number reached 70. Thus, it appeared impossible to adequately separate the disruptive patients from the others, so we had to rely more frequently on sleeping medications than would have been necessary under better circumstances, in order to establish some quiet during the night. Most often, sleeping medications were naturally administered on the surveillance ward, since it was here that those patients resided who most needed sedation both for themselves and for their surrounding environment. This situation is made very clear in the following summary, which cites the percentage of patients in the entire clinic and the percentage of patients on the surveillance ward who, on average, received daily sleeping medication. Opium treatment for anxious patients is not included here.

	Time Periods (I–V)				
	I	II	III	IV	V
Men					
Sleeping Medication Total (pct)	7.42	8.50	10.60	12.39	9.85
On the Surveillance Ward	12.71	14.48	18.52	23.08	15.56
Percentage of Agitated Patients	6.24	7.08	10.16	9.43	9.59
Women					
Sleeping Medication Total (pct)	12.12	9.15	11.56	10.82	14.05
On the Surveillance Ward	14.59	11.63	13.06	12.11	15.20
Percentage of Agitated Patients	22.15	20.09	21.26	24.15	34.70

Without question, the proportion of patients who received sleeping medication is greater on the surveillance ward than throughout the entire clinic, but this difference is everywhere more pronounced on the male side. From this, we can conclude that the administration of sleeping medication to men was essentially rooted in the effort to be able to care for disruptive patients with the least amount of injury to their neighbors on the surveillance ward; whereas for the women, the need for sleeping medication appears hardly lower in the other wards and accords with the large number of agitated elements in general. . . .

The following table provides information about the types of medication administered, in which, though, a number of medications (duboisin, chloral hydrate, chloral morphine) are not cited. Alcohol that is used more frequently as a sleep aid in cases of mild anxiety and fatigue psychosis is also not taken into account, while habitual consumption without medical justification was completely eliminated. The figures refer to daily doses; when it comes to bromine and opium, the unit is generally the sum of a number of separate doses during the day.

	Trional	Sulfate	Hyoscine	Morphine	Opium	Bromine
Men	347	721	20	—	420	279
Women	228	511	206	74	509	312

Among men, actual sleeping medications, trional and sulfate, predominate, which can be explained by the need to establish quiet during the night in over-crowded wards. By contrast, among the women, the experimental administration of the quick-acting and powerful narcotics hyoscine and morphine was noticeably common. In these cases, then, it involved combating sudden bouts of agitation as effectively as possible. The correspondence of this result with our earlier table is self-evident, though it should not be overlooked that in the selection of medication, the personal inclinations and experiences of the ward physician naturally plays a very important role. The more substantial use of (almost always) methodically administered opium and bromine on women is obviously tied to the previously established greater frequency of depressive conditions among the female sex.

From Emil Kraepelin, "Über die Wachabteilung der Heidelberger Irrenklinik," *Allgemeine Zeitschrift für Psychiatrie und psychisch-gerichtliche Medizin* 51 (1895): 1–21. Translated by Greg Eghigian.

Sigmund Freud
(1856–1939)
"The Origin and Development of Psychoanalysis"
(1910)

Sigmund Freud studied neurology at the University of Vienna. Soon after receiving his medical degree, in 1885, he went to Paris to observe the famous Jean-Martin Charcot. Leaving Paris in 1886, Freud returned to Vienna with an interest in the psychological origins of hysteria. He soon established his own private practice and a partnership with his colleague Josef Breuer (1842–1925). Together the two men developed the outlines of a new approach to psychiatric disorders, stressing the roles of memory, trauma, and therapeutic talk. Freud and Breuer soon parted ways, and Freud went on to elaborate his own school of thought—psychoanalysis.

In 1909, Freud came to the United States—his only visit—at the invitation of the president of Clark University, the psychologist G. Stanley Hall. There, Freud gave a series of lectures intended to introduce Americans to the new field. The warm reception he received was a harbinger of things to come, as the United States proved to be far more receptive to psychoanalytic thinking than did his home continent of Europe.

First Lecture

Ladies and Gentlemen: It is a new and somewhat embarrassing experience for me to appear as lecturer before students of the New World. I assume that I owe this honor to the association of my name with the theme of psychoanalysis, and consequently it is of psychoanalysis that I shall aim to speak. I shall attempt to give you in very brief form an historical survey of the origin and further development of this new method of research and cure.

Granted that it is a merit to have created psychoanalysis, it is not my merit. I was a student, busy with the passing of my last examinations, when another physician of Vienna, Dr. Joseph Breuer, made the first application of this method to the case of an hysterical girl (1880–82). We must now examine the history of this case and its treatment, which can be found in detail in *Studien über Hysterie*, later published by Dr. Breuer and myself.

But first one word. I have noticed, with considerable satisfaction, that the majority of my hearers do not belong to the medical profession. Now do not fear that a medical education is necessary to follow what I shall have to say. We shall now accompany the doctors a little way, but soon we shall take leave of them and follow Dr. Breuer on a way which is quite his own.

Dr. Breuer's patient was a girl of twenty-one, of a high degree of intelligence. She had developed in the course of her two years' illness a series of physical and mental disturbances which well deserved to be taken seriously. She had a severe paralysis of both right extremities, with anasthesia, and at times the same affection of the members of the left side of the body; disturbance of eye-movements, and much impairment of vision; difficulty in maintaining the position of the head; an intense Tussis nervosa; nausea when she attempted to take nourishment; and at one time for several weeks a loss of the power to drink, in spite of tormenting thirst. Her power of speech was also diminished, and this progressed so far that she could neither speak nor understand her mother tongue; and, finally, she was subject to states of "absence," of confusion, delirium, alteration of her whole personality. These states will later claim our attention.

When one hears of such a case, one does not need to be a physician to incline to the opinion that we are concerned here with a serious injury, probably of the brain, for which there is little hope of cure and which will probably lead to the early death of the patient. The doctors will tell us, however, that in one type of cases with just as unfavorable symptoms, another, far more favorable, opinion is justified. When one finds such a series of symptoms in the case of a young girl, whose vital organs (heart, kidneys) are shown by objective tests to be normal, but who has suffered from strong emotional disturbances, and when the symptoms differ in certain finer characteristics from what one might logically expect, in a case like this the doctors are not too much disturbed. They consider that there is present no organic lesion of the brain, but that enigmatical state, known since the time of the Greek physicians as hysteria, which can simulate a whole series of

symptoms of various diseases. They consider in such a case that the life of the patient is not in danger and that a restoration to health will probably come about of itself. The differentiation of such an hysteria from a severe organic lesion is not always very easy. But we do not need to know how a differential diagnosis of this kind is made; you may be sure that the case of Breuer's patient was such that no skillful physician could fail to diagnose an hysteria. We may also add a word here from the history of the case. The illness first appeared while the patient was caring for her father, whom she tenderly loved, during the severe illness which led to his death, a task which she was compelled to abandon because she herself fell ill.

So far it has seemed I best to go with the doctors, but we shall soon part company with them. You must not think that the outlook of a patient with regard to medical aid is essentially bettered when the diagnosis points to hysteria rather than to organic disease of the brain. Against the serious brain diseases medical skill is in most cases powerless, but also in the case of hysterical affections the doctor can do nothing. He must leave it to benign nature, when and how his hopeful prognosis will be realized. Accordingly, with the recognition of the disease as hysteria, little is changed in the situation of the patient, but there is a great change in the attitude of the doctor. We can observe that he acts quite differently toward hystericals than toward patients suffering from organic diseases. He will not bring the same interest to the former as to the latter, since their suffering is much less serious and yet seems to set up the claim to be valued just as seriously.

But there is another motive in this action. The physician, who through his studies has learned so much that is hidden from the laity, can realize in his thought the causes and alterations of the brain disorders in patients suffering from apoplexy or dementia, a representation which must be right up to a certain point, for by it he is enabled to understand the nature of each symptom. But before the details of hysterical symptoms, all his knowledge, his anatomical-physiological and pathological education, desert him. He cannot understand hysteria. He is in the same position before it as the layman. And that is not agreeable to any one, who is in the habit of setting such a high valuation upon his knowledge. Hystericals, accordingly, tend to lose his sympathy; he considers them persons who overstep the laws of his science, as the orthodox regard heretics; he ascribes to them all possible evils, blames them for exaggeration and intentional deceit, "simulation," and he punishes them by withdrawing his interest.

Now Dr. Breuer did not deserve this reproach in this case; he gave his patient sympathy and interest, although at first he did not understand how to help her. Probably this was easier for him on account of those superior qualities of the patient's mind and character, to which he bears witness in his account of the case.

His sympathetic observation soon found the means which made the first help possible. It had been noticed that the patient, in her states of "absence," of psychic alteration, usually mumbled over several words to herself. These seemed to spring from associations with which her thoughts were busy. The doctor, who was able to get these words, put her in a sort of hypnosis and repeated them to her over and over, in order to bring up any associations that they might have. The patient yielded to his suggestion and reproduced for him those psychic creations which controlled her thoughts during her "absences," and which betrayed themselves in these single spoken words. These were fancies, deeply sad, often poetically beautiful, day dreams, we might call them, which commonly took as their starting point the situation of a girl beside the sick-bed of her father. Whenever she had related a number of such fancies, she was, as it were, freed and restored to her normal mental life. This state of health would last for several hours, and then give place on the next day to a new "absence," which was removed in the same way by relating the newly-created fancies. It was impossible not to get the impression that the psychic alteration which was expressed in the "absence" was a consequence of the excitations originating from these intensely emotional fancy-images. The patient herself, who at this time of her illness strangely enough understood and spoke only English, gave this new kind of treatment the name "talking cure," or jokingly designated it as "chimney sweeping."

The doctor soon hit upon the fact that through such cleansing of the soul more could be accomplished than a temporary removal of the constantly recurring mental "clouds." Symptoms of the disease would disappear when in hypnosis the patient could be made to remember the situation and the associative connections under which they first appeared, provided free vent was given to the emotions which they aroused. "There was in the summer a time of intense heat, and the patient had suffered very much from thirst; for, without any apparent reason, she had suddenly become unable to drink. She would take a glass of water in her hand, but as soon as it touched her lips she would push it away as though suffering from hydro-

phobia. Obviously for these few seconds she was in her absent state. She ate only fruit, melons and the like, in order to relieve this tormenting thirst. When this had been going on about six weeks, she was talking one day in hypnosis about her English governess, whom she disliked, and finally told, with every sign of disgust, how she had come into the room of the governess, and how that lady's little dog, that she abhorred, had drunk out of a glass. Out of respect for the conventions the patient had remained silent. Now, after she had given energetic expression to her restrained anger, she asked for a drink, drank a large quantity of water without trouble, and woke from hypnosis with the glass at her lips. The symptom thereupon vanished permanently."

Permit me to dwell for a moment on this experience. No one had ever cured an hysterical symptom by such means before, or had come so near understanding its cause. This would be a pregnant discovery if the expectation could be confirmed that still other, perhaps the majority of symptoms, originated in this way and could be removed by the same method. Breuer spared no pains to convince himself of this and investigated the pathogenesis of the other more serious symptoms in a more orderly way. Such was indeed the case; almost all the symptoms originated in exactly this way, as remnants, as precipitates, if you like, of affectively-toned experiences, which for that reason we later called "psychic traumata." The nature of the symptoms became clear through their relation to the scene which caused them. They were, to use the technical term, "determined" (*determiniert*) by the scene whose memory traces they embodied, and so could no longer be described as arbitrary or enigmatical functions of the neurosis.

Only one variation from what might be expected must be mentioned. It was not always a single experience which occasioned the symptom, but usually several, perhaps many similar, repeated traumata cooperated in this effect. It was necessary to repeat the whole series of pathogenic memories in chronological sequence, and of course in reverse order, the last first and the first last. It was quite impossible to reach the first and often most essential trauma directly, without first clearing away those coming later.

You will of course want to hear me speak of other examples of the causation of hysterical symptoms beside this of inability to drink on account of the disgust caused by the dog drinking from the glass. I must, however, if I hold to my programme, limit myself to very few examples. Breuer relates,

for instance, that his patient's visual disturbances could be traced back to external causes, in the following way. "The patient, with tears in her eyes, was sitting by the sick-bed when her father suddenly asked her what time it was. She could not see distinctly, strained her eyes to see, brought the watch near her eyes so that the dial seemed very large (macropia and strabismus conv.), or else she tried hard to suppress her tears, so that the sick man might not see them."

All the pathogenic impressions sprang from the time when she shared in the care of her sick father. "Once she was watching at night in the greatest anxiety for the patient, who was in a high fever, and in suspense, for a surgeon was expected from Vienna, to operate on the patient. Her mother had gone out for a little while, and Anna sat by the sick-bed, her right arm hanging over the back of her chair. She fell into a revery [*sic*] and saw a black snake emerge, as it were, from the wall and approach the sick man as though to bite him. (It is very probable that several snakes had actually been seen in the meadow behind the house, that she had already been frightened by them, and that these former experiences furnished the material for the hallucination). She tried to drive off the creature, but was as though paralyzed. Her right arm, which was hanging over the back of the chair, had 'gone to sleep,' become anasthetic [*sic*] and paretic, and as she was looking at it, the fingers changed into little snakes with deaths-heads. (The nails). Probably she attempted to drive away the snake with her paralyzed right hand, and so the anasthesia [*sic*] and paralysis of this member formed associations with the snake hallucination. When this had vanished, she tried in her anguish to speak, but could not. She could not express herself in any language, until finally she thought of the words of an English nursery song, and thereafter she could think and speak only in this language." When the memory of this scene was revived in hypnosis the paralysis of the right arm, which had existed since the beginning of the illness, was cured and the treatment ended.

When, a number of years later, I began to use Breuer's researches and treatment on my own patients, my experiences completely coincided with his. In the case of a woman of about forty, there was a tic, a peculiar smacking noise which manifested itself whenever she was laboring under any excitement, without any obvious cause. It had its origin in two experiences which had this common element, that she attempted to make no noise, but that by a sort of counter-will this noise broke the stillness. On the first

occasion, she had finally after much trouble put her sick child to sleep, and she tried to be very quiet so as not to awaken it. On the second occasion, during a ride with both her children in a thunderstorm the horses took fright, and she carefully avoided any noise for fear of frightening them still more. I give this example instead of many others which are cited in the "*Studien über Hysterie*."

Ladies and gentlemen, if you will permit me to generalize, as is indispensable in so brief a presentation, we may express our results up to this point in the formula: Our hysterical patients suffer from reminiscences. Their symptoms are the remnants and the memory symbols of certain (traumatic) experiences.

A comparison with other memory symbols from other sources will perhaps enable us better to understand this symbolism. The memorials and monuments with which we adorn our great cities, are also such memory symbols. If you walk through London you will find before one of the greatest railway stations of the city a richly decorated Gothic pillar"—"Charing Cross." One of the old Plantagenet kings, in the thirteenth century, caused the body of his beloved queen Eleanor to be borne to Westminster, and had Gothic crosses erected at each of the stations where the coffin was set down. Charing Cross is the last of these monuments, which preserve the memory of this sad journey. In another part of the city, you will see a high pillar of more modern construction, which is merely called "the monument." This is in memory of the great fire which broke out in the neighborhood in the year 1666, and destroyed a great part of the city. These monuments are memory symbols like the hysterical symptoms; so far the comparison seems justified. But what would you say to a Londoner who today stood sadly before the monument to the funeral of Queen Eleanor, instead of going about his business with the haste engendered by modern industrial conditions, or rejoicing with the young queen of his own heart? Or to another, who before the "Monument" bemoaned the burning of his loved native city, which long since has arisen again so much more splendid than before?

Now hystericals and all neurotics behave like these two unpractical Londoners, not only in that they remember the painful experiences of the distant past, but because they are still strongly affected by them. They cannot escape from the past and neglect present reality in its favor. This fixation of the mental life on the pathogenic traumata is an essential, and

practically a most significant characteristic of the neurosis. I will willingly concede the objection which you are probably formulating, as you think over the history of Breuer's patient. All her traumata originated at the time when she was caring for her sick father, and her symptoms could only be regarded as memory symbols of his sickness and death. They corresponded to mourning, and a fixation on thoughts of the dead so short a time after death is certainly not pathological, but rather corresponds to normal emotional behavior. I concede this: there is nothing abnormal in the fixation of feeling on the trauma shown by Breuer's patient. But in other cases, like that of the tic that I have mentioned, the occasions for which lay ten and fifteen years back, the characteristic of this abnormal clinging to the past is very clear, and Breuer's patient would probably have developed it, if she had not come under the "cathartic treatment" such a short time after the traumatic experiences and the beginning of the disease.

We have so far only explained the relation of the hysterical symptoms to the life history of the patient; now by considering two further moments which Breuer observed, we may get a hint as to the processes of the beginning of the illness and those of the cure. With regard to the first, it is especially to be noted that Breuer's patient in almost all pathogenic situations had to suppress a strong excitement, instead of giving vent to it by appropriate words and deeds. In the little experience with her governess' dog, she suppressed, through regard for the conventions, all manifestations of her very intense disgust. While she was seated by her father's sick bed, she was careful to betray nothing of her anxiety and her painful depression to the patient. When, later, she reproduced the same scene before the physician, the emotion which she had suppressed on the occurrence of the scene burst out with especial strength, as though it had been pent up all along. The symptom which had been caused by that scene reached its greatest intensity while the doctor was striving to revive the memory of the scene, and vanished after it had been fully laid bare. On the other hand, experience shows that if the patient is reproducing the traumatic scene to the physician, the process has no curative effect if, by some peculiar chance, there is no development of emotion. It is apparently these emotional processes upon which the illness of the patient and the restoration to health are dependent. We feel justified in regarding "emotion" as a quantity which may become increased, derived and displaced. So we are forced to the conclusion that the patient fell ill because the emotion developed in the patho-

genic situation was prevented from escaping normally, and that the essence of the sickness lies in the fact that these "imprisoned" (*dingeklemmt*) emotions undergo a series of abnormal changes. In part they are preserved as a lasting charge and as a source of constant disturbance in psychical life; in part they undergo a change into unusual bodily innervations and inhibitions, which present themselves as the physical symptoms of the case. We have coined the name "hysterical conversion" for the latter process. Part of our mental energy is, under normal conditions, conducted off by way of physical innervation and gives what we call "the expression of emotions." Hysterical conversion exaggerates this part of the course of a mental process which is emotionally colored; it corresponds to a far more intense emotional expression, which finds outlet by new paths. If a stream flows in two channels, an overflow of one will take place as soon as the current in the other meets with an obstacle.

You see that we are in a fair way to arrive at a purely psychological theory of hysteria, in which we assign the first rank to the affective processes. A second observation of Breuer compels us to ascribe to the altered condition of consciousness a great part in determining the characteristics of the disease. His patient showed many sorts of mental states, conditions of "absence," confusion and alteration of character, besides her normal state. In her normal state she was entirely ignorant of the pathogenic scenes and of their connection with her symptoms. She had forgotten those scenes, or at any rate had dissociated them from their pathogenic connection. When the patient was hypnotized, it was possible, after considerable difficulty, to recall those scenes to her memory, and by this means of recall the symptoms were removed. It would have been extremely perplexing to know how to interpret this fact, if hypnotic practice and experiments had not pointed out the way. Through the study of hypnotic phenomena, the conception, strange though it was at first, has become familiar, that in one and the same individual several mental groupings are possible, which may remain relatively independent of each other, "know nothing" of each other, and which may cause a splitting of consciousness along lines which they lay down. Cases of such a sort, known as "double personality" ("double conscience"), occasionally appear spontaneously. If in such a division of personality consciousness remains constantly bound up with one of the two states, this is called the conscious mental state, and the other the unconscious. In the well-known phenomena of so-called post hypnotic suggestion, in which a

command given in hypnosis is later executed in the normal state as though by an imperative suggestion, we have an excellent basis for understanding how the unconscious state can influence the conscious, although the latter is ignorant of the existence of the former. In the same way it is quite possible to explain the facts in hysterical cases. Breuer came to the conclusion that the hysterical symptoms originated in such peculiar mental states, which he called "hypnoidal states" (*hypnoide Zustände*). Experiences of an emotional nature, which occur during such hypnoidal states easily become pathogenic, since such states do not present the conditions for a normal draining off of the emotion of the exciting processes. And as a result there arises a peculiar product of this exciting process, that is, the symptom, and this is projected like a foreign body into the normal state. The latter has, then, no conception of the significance of the hypnoidal pathogenic situation. Where a symptom arises, we also find an amnesia, a memory gap, and the filling of this gap includes the removal of the conditions under which the symptom originated.

I am afraid that this portion of my treatment will not seem very clear, but you must remember that we are dealing here with new and difficult views, which perhaps could not be made much clearer. This all goes to show that our knowledge in this field is not yet very far advanced. Breuer's idea of the hypnoidal states has, moreover, been shown to be superfluous and a hindrance to further investigation, and has been dropped from present conceptions of psychoanalysis. Later I shall at least suggest what other influences and processes have been disclosed besides that of the hypnoidal states, to which Breuer limited the causal moment.

You have probably also felt, and rightly, that Breuer's investigations gave you only a very incomplete theory and insufficient explanation of the phenomena which we have observed. But complete theories do not fall from Heaven, and you would have had still greater reason to be distrustful, had any one offered you at the beginning of his observations a well-rounded theory, without any gaps; such a theory could only be the child of his speculations and not the fruit of an unprejudiced investigation of the facts.

Second Lecture

Ladies and Gentlemen: At about the same time that Breuer was using the "talking-cure" with his patient, M. Charcot began in Paris, with the hys-

tericals of the Salpêtrière, those researches which were to lead to a new understanding of the disease. These results were, however, not yet known in Vienna. But when about ten years later Breuer and I published our preliminary communication on the psychic mechanism of hysterical phenomena, which grew out of the cathartic treatment of Breuer's first patient, we were both of us under the spell of Charcot's investigations. We made the pathogenic experiences of our patients, which acted as psychic traumata, equivalent to those physical traumata whose influence on hysterical paralyses Charcot had determined; and Breuer's hypothesis of hypnoidal states is itself only an echo of the fact that Charcot had artificially reproduced those traumatic paralyses in hypnosis.

The great French observer, whose student I was during the years 1885–86, had no natural bent for creating psychological theories. His student, P. Janet, was the first to attempt to penetrate more deeply into the psychic processes of hysteria, and we followed his example, when we made the mental splitting and the dissociation of personality the central points of our theory.* Janet propounds a theory of hysteria which draws upon the principal theories of heredity and degeneration which are current in France. According to his view hysteria is a form of degenerative alteration of the nervous system, manifesting itself in a congenital "weakness" of the function of psychic synthesis. The hysterical patient is from the start incapable of correlating and unifying the manifold of his mental processes, and so there arises the tendency to mental dissociation. If you will permit me to use a banal but clear illustration, Janet's hysterical reminds one of a weak woman who has been shopping, and is now on her way home, laden with packages and bundles of every description. She cannot manage the whole lot with her two arms and her ten fingers, and soon she drops one. When she stoops to pick this up, another breaks loose, and so it goes on.

Now it does not agree very well, with this assumed mental weakness of hystericals, that there can be observed in hysterical cases, besides the phenomena of lessened functioning, examples of a partial increase of functional capacity, as a sort of compensation. At the time when Breuer's patient had forgotten her mother-tongue and all other languages save English, her control of English attained such a level that if a German book was put before

*Editor's note: Freud here refers to Pierre Janet (1859–1947), a French neurologist and psychologist who wrote on hysteria and hypnosis.

her she could give a fluent, perfect translation of its contents at sight. When later I undertook to continue on my own account the investigations begun by Breuer, I soon came to another view of the origin of hysterical dissociation (or splitting of consciousness). It was inevitable that my views should diverge widely and radically, for my point of departure was not, like that of Janet, laboratory researches, but attempts at therapy. Above everything else, it was practical needs that urged me on. The cathartic treatment, as Breuer had made use of it, presupposed that the patient should be put in deep hypnosis, for only in hypnosis was available the knowledge of his pathogenic associations, which were unknown to him in his normal state. Now hypnosis, as a fanciful, and so to speak, mystical, aid, I soon came to dislike; and when I discovered that, in spite of all my efforts, I could not hypnotize by any means all of my patients, I resolved to give up hypnotism and to make the cathartic method independent of it.

Since I could not alter the psychic state of most of my patients at my wish, I directed my efforts to working with them in their normal state. This seems at first sight to be a particularly senseless and aimless undertaking. The problem was this: to find out something from the patient that the doctor did not know and the patient himself did not know. How could one hope to make such a method succeed? The memory of a very noteworthy and instructive proceeding came to my aid, which I had seen in Bernheim's clinic at Nancy. Bernheim showed us that persons put in a condition of hypnotic somnambulism, and subjected to all sorts of experiences, had only apparently lost the memory of those somnambulic experiences, and that their memory of them could be awakened even in the normal state.* If he asked them about their experiences during somnambulism, they said at first that they did not remember, but if he persisted, urged, assured them that they did know, then every time the forgotten memory came back.

Accordingly I did this with my patients. When I had reached in my procedure with them a point at which they declared that they knew nothing more, I would assure them that they did know, that they must just tell it out, and I would venture the assertion that the memory which would emerge at the moment that I laid my hand on the patient's forehead would be the right one. In this way I succeeded, without hypnosis, in learning

*Editor's note: Hippolyte Bernheim (1840–1919) was known for his contention that hypnosis was little more than a form of ritualized suggestion.

from the patient all that was necessary for a construction of the connection between the forgotten pathogenic scenes and the symptoms which they had left behind. This was a troublesome and in its length an exhausting proceeding, and did not lend itself to a finished technique. But I did not give it up without drawing definite conclusions from the data which I had gained. I had substantiated the fact that the forgotten memories were not lost. They were in the possession of the patient, ready to emerge and form associations with his other mental content, but hindered from becoming conscious, and forced to remain in the unconscious by some sort of a force. The existence of this force could be assumed with certainty, for in attempting to drag up the unconscious memories into the consciousness of the patient, in opposition to this force, one got the sensation of his own personal effort striving to overcome it. One could get an idea of this force, which maintained the pathological situation, from the resistance of the patient.

It is on this idea of resistance that I based my theory of the psychic processes of hystericals. It had been found that in order to cure the patient it was necessary that this force should be overcome. Now with the mechanism of the cure as a starting point, quite a definite theory could be constructed. These same forces, which in the present situation as resistances opposed the emergence of the forgotten ideas into consciousness, must themselves have caused the forgetting, and repressed from consciousness the pathogenic experiences. I called this hypothetical process "repression" (*Verdrängung*), and considered that it was proved by the undeniable existence of resistance.

But now the question arose: what were those forces, and what were the conditions of this repression, in which we were now able to recognize the pathogenic mechanism of hysteria? A comparative study of the pathogenic situations, which the cathartic treatment has made possible, allows us to answer this question. In all those experiences, it had happened that a wish had been aroused, which was in sharp opposition to the other desires of the individual, and was not capable of being reconciled with the ethical, aesthetic and personal pretensions of the patient's personality. There had been a short conflict, and the end of this inner struggle was the repression of the idea which presented itself to consciousness as the bearer of this irreconcilable wish. This was, then, repressed from consciousness and forgotten. The incompatibility of the idea in question with the "ego" of the patient

was the motive of the repression, the ethical and other pretensions of the individual were the repressing forces. The presence of the incompatible wish, or the duration of the conflict, had given rise to a high degree of mental pain; this pain was avoided by the repression. This latter process is evidently in such a case a device for the protection of the personality.

I will not multiply examples, but will give you the history of a single one of my cases, in which the conditions and the utility of the repression process stand out clearly enough. Of course for my purpose I must abridge the history of the case and omit many valuable theoretical considerations. It is that of a young girl, who was deeply attached to her father, who had died a short time before, and in whose care she had shared—a situation analogous to that of Breuer's patient. When her older sister married, the girl grew to feel a peculiar sympathy for her new brother-in-law, which easily passed with her for family tenderness. This sister soon fell ill and died, while the patient and her mother were away. The absent ones were hastily recalled, without being told fully of the painful situation. As the girl stood by the bedside of her dead sister, for one short moment there surged up in her mind an idea, which might be framed in these words: "Now he is free and can marry me." We may be sure that this idea, which betrayed to her consciousness her intense love for her brother-in-law, of which she had not been conscious, was the next moment consigned to repression by her revolted feelings. The girl fell ill with severe hysterical symptoms, and, when I came to treat the case, it appeared that she had entirely forgotten that scene at her sister's bedside and the unnatural, egoistic desire which had arisen in her. She remembered it during the treatment, reproduced the pathogenic moment with every sign of intense emotional excitement, and was cured by this treatment.

Perhaps I can make the process of repression and its necessary relation to the resistance of the patient, more concrete by a rough illustration, which I will derive from our present situation.

Suppose that here in this hall and in this audience, whose exemplary stillness and attention I cannot sufficiently commend, there is an individual who is creating a disturbance, and, by his ill-bred laughing, talking, by scraping his feet, distracts my attention from my task. I explain that I cannot go on with my lecture under these conditions, and thereupon several strong men among you get up, and, after a short struggle, eject the disturber of the peace from the hall. He is now "repressed," and I can continue

my lecture. But in order that the disturbance may not be repeated, in case the man who has just been thrown out attempts to force his way back into the room, the gentlemen who have executed my suggestion take their chairs to the door and establish themselves there as a "resistance," to keep up the repression. Now, if you transfer both locations to the psyche, calling this "consciousness," and the outside the "unconscious," you have a tolerably good illustration of the process of repression.

We can see now the difference between our theory and that of Janet. We do not derive the psychic fission from a congenital lack of capacity on the part of the mental apparatus to synthesize its experiences, but we explain it dynamically by the conflict of opposing mental forces, we recognize in it the result of an active striving of each mental complex against the other.

New questions at once arise in great number from our theory. The situation of psychic conflict is a very frequent one; an attempt of the ego to defend itself from painful memories can be observed everywhere, and yet the result is not a mental fission. We cannot avoid the assumption that still other conditions are necessary, if the conflict is to result in dissociation. I willingly concede that with the assumption of "repression" we stand, not at the end, but at the very beginning of a psychological theory. But we can advance only one step at a time, and the completion of our knowledge must await further and more thorough work.

Now do not attempt to bring the case of Breuer's patient under the point of view of repression. This history cannot be subjected to such an attempt, for it was gained with the help of hypnotic influence. Only when hypnosis is excluded can you see the resistances and repressions and get a correct idea of the pathogenic process. Hypnosis conceals the resistances and so makes a certain part of the mental field freely accessible. By this same process the resistances on the borders of this field are heaped up into a rampart, which makes all beyond inaccessible.

The most valuable things that we have learned from Breuer's observations were his conclusions as to the connection of the symptoms with the pathogenic experiences or psychic traumata, and we must not neglect to evaluate this result properly from the standpoint of the repression-theory. It is not at first evident how we can get from the repression to the creation of the symptoms. Instead of giving a complicated theoretical derivation, I will return at this point to the illustration which I used to typify repression.

Remember that with the ejection of the rowdy and the establishment of the watchers before the door, the affair is not necessarily ended. It may very well happen that the ejected man, now embittered and quite careless of consequences, gives us more to do. He is no longer among us, we are free from his presence, his scornful laugh, his half-audible remarks, but in a certain sense the repression has miscarried, for he makes a terrible uproar outside, and by his outcries and by hammering on the door with his fists interferes with my lecture more than before. Under these circumstances it would be hailed with delight if possibly our honored president, Dr. Stanley Hall, should take upon himself the role of peacemaker and mediator. He would speak with the rowdy on the outside, and then turn to us with the recommendation that we let him in again, provided he would guarantee to behave himself better. On Dr. Hall's authority we decide to stop the repression, and now quiet and peace reign again. This is in fact a fairly good presentation of the task devolving upon the physician in the psychoanalytic therapy of neuroses. To say the same thing more directly: we come to the conclusion, from working with hysterical patients and other neurotics, that they have not fully succeeded in repressing the idea to which the incompatible wish is attached. They have, indeed, driven it out of consciousness and out of memory, and apparently saved themselves a great amount of psychic pain, but in the unconscious the suppressed wish still exists, only waiting for its chance to become active, and finally succeeds in sending into consciousness, instead of the repressed idea, a disguised and unrecognizable surrogate-creation (*Ersatzbildung*), to which the same painful sensations associate themselves that the patient thought he was rid of through his repression. This surrogate of the suppressed idea—the symptom—is secure against further attacks from the defences of the ego, and instead of a short conflict there originates now a permanent suffering. We can observe in the symptom, besides the tokens of its disguise, a remnant of traceable similarity with the originally repressed idea; the way in which the surrogate is built up can be discovered during the psychoanalytic treatment of the patient, and for his cure the symptom must be traced back over the same route to the repressed idea. If this repressed material is once more made part of the conscious mental functions—a process which supposes the overcoming of considerable resistance—the psychic conflict which then arises, the same which the patient wished to avoid, is made capable of a happier termination, under the guidance of the physician, than is offered by

repression. There are several possible suitable decisions which can bring conflict and neurosis to a happy end; in particular cases the attempt may be made to combine several of these. Either the personality of the patient may be convinced that he has been wrong in rejecting the pathogenic wish, and he may be made to accept it either wholly or in part; or this wish may itself be directed to a higher goal which is free from objection, by what is called sublimation (*Sublimierung*); or the rejection may be recognized as rightly motivated, and the automatic and therefore insufficient mechanism of repression be reinforced by the higher, more characteristically human mental faculties: one succeeds in mastering his wishes by conscious thought.

Forgive me if I have not been able to present more clearly these main points of the treatment which is to-day known as "psychoanalysis." The difficulties do not lie merely in the newness of the subject.

Regarding the nature of the unacceptable wishes, which succeed in making their influence felt out of the unconscious, in spite of repression; and regarding the question of what subjective and constitutional factors must be present for such a failure of repression and such a surrogate or symptom creation to take place, we will speak in later remarks.

From Sigmund Freud, "The Origin and Development of Psychoanalysis," translated by Henry W. Chase, *American Journal of Psychology* 21 (1910): 181–218.

Vincent "Confessions of an Agoraphobic Victim" (1919)

Since ancient times, observers have been aware that some individuals are plagued by delusional fears. During the second half of the nineteenth century, clinicians began to apply the term *phobia* to a variety of these extremes forms of anxiety. One of the earliest phobias to draw attention was the fear of certain, typically public, spaces. In 1871, the Berlin psychiatrist Carl Westphal (1833–1890),

having encountered patients complaining of an inability to cross or go down particular streets, dubbed this condition *agoraphobia*. Like neurasthenia, agoraphobia appeared to many to be a peculiarly urban phenomenon, and up until World War I, it was diagnosed primarily in men. The following autobiographical description of agoraphobia gives a sense of how the malady was experienced as a form of nervousness around the turn of the century.

For some time I have been planning to commit to writing personal observations of my condition, sensations and experiences during a long period of suffering from a malady which, for lack of a better name, medical men have termed "Agora-phobia"—fear of an open place.

As I am unacquainted with medical literature I do not know how much has been written on this subject. Only one case of "confessions" has come to my notice. Some time ago I read several pamphlets on "Religion and Medicine"—Emmanuel Church publications—one of which discussed various "nervous" disorders, among them the "phobias." In the course of the discussion there was introduced testimony of a man who had been grievously afflicted with agoraphobia. I devoured these "confessions" with the greatest avidity. It was the first and only time I had read any testimony of an individual thus afflicted.

This same pamphlet stated that the man had since become almost cured. Encouraged by these good tidings I tried to practice mental poise and tranquility for several weeks. During those few weeks I saw wonderful improvement in my condition. However, I lapsed gradually into my old habit of self-neglect, and with this neglect improvement in my health ceased.

The testimony taken from the above mentioned pamphlet leads one to infer that the affliction came upon the victim rather suddenly, and that the symptoms of the malady were present only occasionally: namely, when the man was on certain streets and at definite places on these streets.

If the above inference is correct I have to testify that my case is quite different. In the first place, my malady came upon me gradually and went through definite stages of development. Second, I am conscious of my affliction every minute that I am awake.

I am now in mid-life and I have not seen a well day since I was about twelve years of age. Before I experienced any of the symptoms of agorapho-

bia I recall that a strange affliction came over me, an affliction that seemed to baffle the country doctors who were consulted. I was taken suddenly with "spells" which lasted about thirty minutes. During these attacks I was entirely conscious and rational. As I remember the affliction, a sort of chill came over me—not like an ordinary chill, but a sort of "coldness" that produced a very unusual sensation, or perhaps, a lack of sensation would describe it more accurately. I have an impression that the physicians suggested that it might possibly have been due to a temporary stoppage of circulation. At any rate the remedy applied was vigorous rubbing of my body with rough towels, or with the bare hands by those attending me; some times, when it was convenient, a hot bath was resorted to; stimulants were also administered. I was more liable to these attacks during times of excitement. For instance, I recall that one of the worst attacks I ever had came over me while I was attending the funeral of a relative. When there seemed to be no outward cause that brought on the attacks, it was noted by my parents that they recurred periodically; I believe it was every fourteen days or was it eleven days?

My own belief now is that the illness referred to was due to some disorder of the nervous system. At any rate, after a few months I outgrew the tendency. However, I was not a well boy. I was abnormally timid and more or less melancholy, and was given to worry and brooding.

In this connection I would remark that I was born with an active, nervous temperament, and was always, as a boy, more or less timid. I was abnormally alert when there was possibility of danger. I remember how I used to run and leap like a fawn when passing through tall grass and weeds in the summer time, fearing lest I should encounter snakes, which were rather common.

When my strange illness came upon me I worried over it, fearing that I should die in one of the attacks. During this overwrought state of mind I was much affected by a terrible tragedy which took place in our community. One of my playmates, a boy about eleven years of age, disappeared one day. The supposition was that he had fallen into the river and had been drowned. On this theory, the river was dragged day after day for more than a week, but no body was discovered. Then the theory was advanced that he had been kidnapped, or perhaps, he being a venturesome lad, had started out to "see the world" and had met with foul play. I recall very well how the entire community was stirred and what effect it had on the boys and girls

of the village and neighborhood. Finally, one morning in late autumn, the body of my playmate was found on the bank of the river, at a bend in the stream just below the village. But he had not been drowned—his throat was cut from ear to ear. The murder had been committed in a cellar in the village by a half-crazed woman who had later carried the body, by night, and placed it on the bank of the river.

This whole affair had a most depressing effect upon me. After that I almost feared to be alone, was afraid to go to the barn in the day time, and suffered when put to bed in the dark. Perhaps the worry over my illness, together with the depressing effect of the tragedy, brought on a severe attack of nervous dyspepsia. This resulted in malnutrition for my over-sensitive nervous system which, perhaps, laid the foundation for the "phobias."

It was during the months which followed that I remember having experienced the first symptoms of agoraphobia. There was a high hill not far from my home in the country where we boys used to coast in the winter time. One evening while coasting, in company with other boys of the neighborhood, I experienced an uncomfortable feeling each time we returned to the top of the hill. It was not a well defined symptom of this horrible (I use the term deliberately) malady, but later experiences have taught me that it possessed the unmistakable earmarks. As the months went by the symptoms developed, with the result that I avoided hill-tops, so far as possible.

Later, perhaps a year or so, I commenced having a dread of wide fields, especially when the fields consisted of pasture land and were level, with the grass cropped short like the grass on a well-kept lawn. I likewise commenced to dread high things, and especially to ascend anything high. I even had a fear of crowds of people, and later of wide streets and parks.

I have outgrown the fear of crowds largely, but an immense building or a high rocky bluff fills me with dread. However the architecture of the building has much to do with the sort of sensation produced. Ugly architecture greatly intensifies the fear.

In this connection I would remark that I have come to wonder if there is real art in many of the so-called "improvements" in some of our cities, for, judging from the effect they produce on me, they constitute bad art.

But the one thing that I would make plain is that the malady is always present. As I write in my study I am painfully conscious of it—in fact, I am

conscious of it during every hour that I am awake. The fear, intensified, that comes over me while crossing a wide street is, it seems to me, an outcropping of a permanent condition.

It is not pain that I feel, but it seems to me that it is more than a dread. I am not nervous, as some people whom I know—I mean in the same way, but it certainly is a case of "nerves." Let me illustrate:—I enter a home and sit in an arm-chair chatting with my friend; I soon find myself gripping the arm of the chair with each hand. My toes curl in my shoes, and there is a sort of tenseness all over my muscles.

At times my phobias are much more pronounced than at other times. Sometimes, after a strenuous day, on the following morning, I find myself almost dreading to walk across a room; at other times I can cross a street without any pronounced discomfort. Manual labor improves my condition. Walking and riding horseback are beneficial.

Usually I feel better in the evening than in the morning, partly because the darkness seems to have a quieting effect upon me. I love a snow storm a regular blizzard, and feel much less discomfort in going about the town or riding on a train on such days, probably because one's view is obstructed. In fact I welcome stormy days, strange to say, with a zest that is hard to appreciate; in short, some of the most stormy days of the hard winters of this region stand out as bright spots in my life. On such days I make it a point to be out and about the town.

I dread going out on water in a boat, especially if the surface is smooth; I much prefer to have the waves rolling high. The most restful place in all the world for me is in a wood, where there is much variety in the trees and plenty of underbrush, with here and there low hills and little valleys, and especially along a winding brook. I love "the quiet places by the woods." Also the little lakes with their narrow bays and wooded shoreline. I love quiet, restful landscape. It seems to rest my eyes and soothe my spirit. On the other hand, let the landscape be bold and rugged and bleak and it strikes terror to my soul.

I lived in New Haven during four years, while a student at Yale, and never climbed to the top of East Rock. And the big Green near the university always made me very uncomfortable when I looked at it.

I ride a bicycle along streets with comparative comfort where I should suffer agony were I to walk. In walking I feel less uncomfortable in passing along the street if I carry a suit-case or traveling-bag—something to grip.

When I think of the agony which I have experienced for many years I am astounded at the endurance of the human spirit. Let me illustrate:—I have such a dread of crossing a long bridge on foot that it would require more courage for me to walk to the part of my town situated across the river than it would to face a nest of Boche machine guns. And yet day after day, month after month, and year after year I have carried in my soul the dread of such an eventuality.

No one knows the truth about my condition. It is one of the characteristics of the victim of the disease to conceal it most cunningly. I think I am an honest man in all essential things. My credit is good at the banks. But I have deliberately told lies to avoid embarrassing situations and have even changed my plans to have my lies "come true."

I have never mentioned my condition to a physician. I have passed several examinations for life-insurance policies—in fact, have never been refused a policy by any life insurance company. I eat and sleep well, am rather strong and wiry physically. My occupation makes heavy demands on the vitality and entails considerable mental exercise, but I am seldom unable to take up my duties on account of indisposition. However, in my own mind I am a nervous wreck, weak, worthless, and unworthy of the high respect which the community accords me.

In spite of all this I seem to exercise marked power of leadership in my town, and am known as a public speaker of ability.

Of course, the paramount question with me is: Is there hope of a cure? Can I ever take my place in the world unhandicapped as other men are, and enjoy a single day undepressed by dark dread? If I could be as other men, it seems to me that my usefulness should be increased a hundredfold. Those who have not been thus afflicted cannot understand just what I mean.

I see a man hobbling past my house on crutches, a cripple for life, and I actually envy him. At times I would gladly exchange places with the humblest day-laborer who walks unafraid across the public square or saunters tranquilly over the viaduct on his way home after the day's work.

From Vincent, "Confessions of an Agoraphobic Victim," *American Journal of Psychology* 30 (1919): 295–299.